

The Zenith Alora Guide

One guide for every team member who uses Alora. Each step is explained in plain words and shown with pictures, so you can do your work with confidence.

If you visit patients, or if you work in the office, the answer you need is in here.

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Waiting for Administrator approval



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0

How to Use This Guide

● Everyone

About 8 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Find the chapters that match your job
- ✓ Understand the signs and labels used in every chapter
- ✓ Read the pictures and follow the numbered parts
- ✓ Use search, quizzes, and the progress tracker

This guide teaches you to use Alora. Alora is the computer system Zenith uses for patient visits, patient records, and billing. You do not need computer experience to start. Each chapter explains one topic in small steps.

Read the chapters for your job first. Then keep this guide close. When you are not sure what to do, look in the contents list or use the search box.

Find your path

Every team member starts with the same five chapters. After that, choose the path for your job. Select a chapter name to jump to it.



Clinicians

RN, LPN, PT, OT, ST, and MSW

1. What Alora is
2. The patient journey
3. Privacy and security
4. Getting started

5. Finding your way
6. Your schedule
7. The visit day
8. Clocking in and out
9. Working without internet
10. Visit notes
11. Signatures and consent
12. For nurses, therapists, and social workers
13. Problem solver
14. Safety and reporting
15. Honest records



Aides

HHA and CNA

1. What Alora is
2. The patient journey
3. Privacy and security
4. Getting started
5. Finding your way
6. Your schedule
7. The visit day
8. Clocking in and out
9. Working without internet
10. Visit notes
11. Signatures and consent
12. For home health aides and CNAs
13. Problem solver
14. Safety and reporting
15. Honest records



Office team

Intake, scheduling, clinical review, billing, and administration

1. What Alora is
2. The patient journey
3. Privacy and security
4. Getting started
5. Finding your way
6. Intake and new patients
7. Scheduling and staffing
8. Live Monitor and EVV exceptions
9. Clinical review and quality
10. Billing and getting paid
11. Daily, weekly, and monthly routines
12. Problem solver
13. Safety and reporting
14. Honest records

Office team members should also read the field chapters (6 to 11). You support the people in the field, and you need to know what their screens look like.

Find it fast

Look for what you want to do. Select the chapter to go there.

I WANT TO...	GO TO
Set up my account and my phone	Chapter 4
Keep patient information private	Chapter 3
Read my schedule, or take an open visit	Chapter 6
Know the steps of a visit	Chapter 7
Clock in, clock out, or fix a clocking mistake	Chapter 8
Work where there is no internet, or sync my work	Chapter 9
Write a visit note, or correct one	Chapter 10

I WANT TO...	GO TO
Get a patient signature	Chapter 11
Do my aide tasks, or report a change in a patient	Chapter 12
Complete an assessment, a plan of care, or a recertification	Chapter 13
Enter a new patient	Chapter 14
Build the schedule or fill an open visit	Chapter 15
Handle a late visit or an EVV exception	Chapter 16
Check records and follow up on signatures	Chapter 17
Send a claim, or work a denial	Chapter 18
Fix a problem, or find out who to call	Chapter 20
Report an emergency, an incident, or abuse	Chapter 21
Know what is never allowed in a record	Chapter 22
Find a phone number or a Zenith deadline	Appendix A
Look up a word	Appendix B

The signs you will see

Colored boxes appear in every chapter. Each one has a picture, a title, and a color. You can understand the box from the title alone. You never need to depend on color.

 Do What you should do.	 Do not What you should avoid.	 Important Something you must not miss.
 Tip An easier or safer way.	 The rule A law or an official rule.	 Time limit A deadline.
 Example A real situation.	 Stop and ask Do not continue	

until you talk to
someone.

Where each rule comes from

Some rules are laws. Some are Zenith rules. Some are simply the best way to work. Each rule in this guide has a label, so you always know which kind it is.

FEDERAL RULE

Federal rule. It comes from the United States government, for example Medicare. Zenith must follow it. It cannot be changed by Zenith.

FLORIDA LAW

Florida law. It comes from the State of Florida. Zenith and every team member must follow it.

ZENITH STANDARD

Zenith standard. It is a rule set by Zenith. The exact numbers, such as times and deadlines, are listed in Appendix A. They are underlined with dots in the text, like this:
the end of the same day.

GOOD PRACTICE

Good practice. It is not a law, but it is the safest and clearest way to work. Follow it unless your supervisor tells you otherwise.

Words we use the same way every time

This guide uses the same word for the same thing. This makes it easier to learn, and easier to read in a second language.

WORDS TO KNOW

Team member

Anyone who works for Zenith, including employees and contractors.

Clinician

A licensed health professional: RN, LPN, PT, OT, ST, or MSW.

Aide

A home health aide (HHA) or certified nursing assistant (CNA).

Office team

Staff who work in the Zenith office: intake, scheduling, clinical review, billing, and administration.

Patient

The person who receives care from Zenith.

Visit

One time a team member goes to a patient to give a service.

Supervisor

The person you report to. For clinical questions this is the Director of Nursing or your clinical supervisor.

Tap and select

On a phone, you **tap** with your finger. On a computer, you **click** with the mouse. This guide says **select** when either one works.

Words with a dotted underline, like EVV, are explained in the Glossary. Hold your finger or mouse on the word to see a short meaning. Select it to open the full meaning.

How to read the pictures

This guide uses simple drawings of phone screens and computer screens. Each drawing has small gold circles with numbers. Find the same number in the list next to the picture. The list tells you what that part of the screen does.



ABOUT THE SCREENS IN THIS GUIDE

Alora is updated by its maker, and each agency can set it up in its own way. The drawings in this guide are simple pictures that show the idea. **Your screen may look different.** If a button has a different name, look for the button that does the same job. If you cannot find it, ask your supervisor. Do not guess.

Learn, check, and keep track

- Every chapter starts with **In this chapter you will learn to**. This is your goal.
- Every chapter ends with **Remember**. This is a short summary you can review in one minute.
- After that, **Check yourself** gives you questions. Select a question to see the answer. Try to answer it in your mind first.
- Select **I finished this chapter** at the end. The progress bar on the left counts your chapters. It is saved only on the device you are using.
- Use the **search box** to find any word. Try *clock out* or *signature*.
- You can print this guide. The print version opens every answer and uses black and white colors that are easy to read.

☆ REMEMBER

- Start with chapters 1 to 5. Then follow the path for your job.
- Every rule has a label: Federal rule, Florida law, Zenith standard, or Good practice.
- The pictures are simple drawings. If your screen is different, look for the button with the same job.
- When you are not sure, ask. Do not guess.

CHECK YOURSELF



A rule in this guide has the label Zenith standard. Where do you find the exact number or time?

Answer: In **Appendix A**. Zenith standards have a dotted underline in the text, and every one is listed in Appendix A.

? Your screen has a button named Begin Visit. The guide says Clock In. What do you do?

Answer: Use the button that does the same job. The names can be different. If you are not sure, ask your supervisor before you tap anything.

PART A

The Basics

These five chapters are for everyone. They explain what Alora is, how a patient moves through Zenith, how to keep information private, and how to set up your account.

1

What Alora Is and Why We Use It

● Everyone

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Say in one sentence what Alora is
- ✓ Name the four jobs Alora does for Zenith
- ✓ Explain what EVV means and why it matters
- ✓ Follow the ten golden rules of Alora

Alora is the computer system Zenith uses to run patient care. We use it to plan visits, prove visits happened, write patient records, and send bills. Every team member uses a part of it.

WORDS TO KNOW

Alora

A cloud-based computer system for home health agencies. It includes scheduling, visit proof, patient records, and billing.

Cloud-based

The information is stored on secure computers on the internet, not only on your phone. You can open it from any approved device.

EMR

Electronic Medical Record. The patient's chart, kept on a computer.

EVV

Electronic Visit Verification. Proof that a visit happened.

App

A program you install on your phone or tablet.

Alora does four jobs

Think of Alora as one place that follows every visit from the plan to the payment. It does four jobs, in this order.

1



Schedule

Plans who visits which patient, and when.



2



Prove the visit

EVV records who, where, and when.



3



Keep the record

Notes, vital signs, orders, and signatures.



4



Get paid

Bills Medicare, Medicaid, and other payers.

1. Schedule

The scheduler puts each visit on the calendar and assigns it to a team member. You see your visits on your phone. Alora can also tell team members about open visits that need someone. Alora shows schedules from the point of view of the patient, the team member, or the whole agency, by day, week, or month.

2. Prove the visit (EVV)

When you arrive, you **clock in**. When you finish, you **clock out**. Alora records the time and the place. This is called EVV. Alora can record your place with your phone's GPS. If you cannot use the app, Alora also offers a telephone method as a backup.

3. Keep the record

After care, you write a visit note. Clinicians also complete assessments and plans of care. Patients and team members sign on the screen. Alora keeps all of this in the patient's electronic record.

4. Get paid

The office uses the visit proof and the notes to bill the payer. If a visit is missing, or a note is not finished, the claim cannot be sent. So your work in steps 2 and 3 pays for the whole agency.

Two ways to use Alora

	THE MOBILE APP	THE COMPUTER (WEB BROWSER)
Who uses it most	Clinicians and aides in the field	The office team
What it is for	Schedule, clock in and out, visit notes, signatures	Intake, scheduling, monitoring, review, billing, reports
Works without internet?	Yes. The app saves your work and sends it later.	No. It needs internet.



WHICH APP DO I USE?

Alora has more than one phone app. You may hear the names **Alora Anywhere** and **Alora Plus**. The office will tell you which one to install. The name is written in Appendix A.

What you can see depends on your job

Alora gives each person a **role**. The role decides what you can see and do. An aide sees the schedule and the tasks. The office sees the billing. This protects patient privacy. If you cannot open something that you need for your work, ask your supervisor. Do not ask another person to open it for you with their account.

Why accurate records matter

There is an old saying in health care: **"If it is not written, it did not happen."** A record is the only proof of the care you gave. It matters for four reasons.



Safety

The next team member reads your note before caring for the same patient.



The law

Medicare, Medicaid, and Florida require complete records.



Payment

Claims are paid only when the record supports them.



Protection

A good record protects the patient, you, and Zenith.

The ten golden rules of Alora

If you remember only one page of this guide, remember these ten rules. Every other chapter explains them in more detail.



1. Use only your own account.

Never share your username or password. Never use another person's account.



2. Clock in and out at the patient's home.

Clock in when care begins. Clock out when care ends. Use the real time.



3. Write the truth.

Write what you saw, did, and heard. Never guess. Never copy an old note.



4. Write it on time.

Finish your notes by the Zenith deadline, **the end of the same day.**



5. Patients sign for themselves.

You can never sign for a patient. You never ask another person to sign for you.



6. Protect privacy.

Open only what you need. Keep screens private. Never share patient information in personal messages or on social media.



7. Send your work.

Make sure everything on your phone is synced before you finish work for the day.



8. Report problems right away.

A late visit, a fall, a missing item, a wrong record. Tell the office at once.



9. Fix mistakes the right way.

Add a correction with the real date and time. Never erase, hide, or backdate.



10. Ask when you are not sure.

A quick question is always better than a wrong record.

☆ REMEMBER

- Alora plans visits, proves visits, keeps the record, and sends the bill.
- EVV means Electronic Visit Verification. It is proof that a visit happened.
- Field staff use the mobile app. The office uses a computer.
- Your role decides what you can see. Use only your own account.
- "If it is not written, it did not happen."

CHECK YOURSELF



What are the four jobs Alora does?

Answer: Schedule visits, prove visits (EVV), keep the patient record, and bill payers (get paid).

? What does EVV stand for, and what does it show?

Answer: Electronic Visit Verification. It shows that a visit happened: who gave the service, who received it, which service, the date, where, and the start and end times.

? A co-worker forgot their password. Can you let them use your account for one visit?

Answer: No. Every person must use only their own account. Sharing an account breaks privacy rules and makes the visit record false. Tell the office so they can reset the password.

? Why does your visit note matter to the billing team?

Answer: Claims are paid only when the record supports the visit. A missing or late note can delay the claim for the whole agency.

2

The Patient Journey and Who Does What

● Everyone

About 5 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Follow one patient from referral to discharge
- ✓ Know which team member does each step
- ✓ See how late work in one step slows every step after it

Every patient follows the same path through Zenith. When you know the path, you can see why your part matters, and who is waiting for your work.

The ten steps of care

- 1 Referral**
A doctor, hospital, or family member asks Zenith to care for a patient. The **office team** receives the request. In Alora, a new patient record is started.
- 2 Intake and checks**
The office confirms the patient's details, the insurance, and the doctor's orders. The office also plans the first visit. *See Chapter 14.*
- 3 Initial assessment visit**
A registered nurse (RN) visits the patient. This is the first look at what the patient needs. The nurse also explains the patient's rights.

FEDERAL RULE

The initial assessment visit must happen within **48 hours** of the referral, or within 48 hours of the patient's return home, or on the date the doctor ordered. (42 CFR 484.55)

4

Start of care and the full assessment

The date of the first visit that begins care is the **start of care**. The clinician finishes the full assessment, called **OASIS**, for Medicare patients.

FEDERAL RULE

The full assessment must be complete no later than **5 calendar days** after the start of care. (42 CFR 484.55)

5

Plan of care

The clinician writes the plan of care. It lists the goals, the services, and how often each team member will visit. The doctor signs it. Every visit follows this plan.

6

Regular visits

Clinicians and aides visit as the plan says. At each visit you clock in, give care, write a note, collect a signature, and clock out. *See Chapters 6 to 12.*

7

Watching and supervising

The office watches visits on the Live Monitor. A nurse supervises aides. The clinical reviewer checks records for missing items. *See Chapters 16 and 17.*

8

Recertification

Near the end of each 60-day period, the clinician checks the patient again and decides if care should continue.

9

Billing

The office bills the payer, using the visit proof, the notes, and the signed orders. *See Chapter 18.*

10

Discharge

When care ends, the clinician completes a discharge record, and the office closes the episode.

Who does what in Alora

This table shows how each group works with each task. **DOES** means you do the task.

CHECKS means you look at it and make sure it is right. **FOLLOWS** means you use it in your work. **NOT YOU** means the task belongs to other people.

TASK IN ALORA	CLINICIANS	AIDES	OFFICE TEAM
Enter a new patient from a referral	NOT YOU	NOT YOU	DOES
Plan and assign visits	FOLLOWS	FOLLOWS	DOES
Clock in and clock out	DOES	DOES	CHECKS
Write a visit note	DOES	DOES	CHECKS
Complete the assessment (OASIS)	DOES (RN or therapist)	NOT YOU	CHECKS
Write the plan of care	DOES	FOLLOWS	CHECKS and sends for signature
Supervise aide care	DOES (RN or therapist)	FOLLOWS	CHECKS dates
Watch visits as they happen	NOT YOU	NOT YOU	DOES
Review EVV problems	EXPLAINS	EXPLAINS	DOES
Bill the payer	NOT YOU	NOT YOU	DOES

Exact permissions can be different for each person. Zenith's Administrator sets them in Alora.

Why late work slows everyone

Each step waits for the step before it. One late note can stop the chain.

1



Note is late

The visit note is not written after the visit.



2



Review waits

The clinical reviewer cannot check the record.



3



Claim waits

Billing cannot send the claim.



4



Payment is late

Zenith receives payment weeks later.



A SMALL DELAY

A nurse finishes a visit at 3:00 PM but writes the note three days later. The reviewer finds a missing vital sign on day 4. The nurse does not remember the exact number. The claim cannot be completed honestly. Writing the note the same day would have prevented all of this.

☆ REMEMBER

- A patient moves through ten steps: referral to discharge.
- The first nurse visit must happen within 48 hours of referral (or the ordered date). The full assessment must be done within 5 calendar days of start of care.
- The office plans and bills. Clinicians and aides deliver care and write the record.
- Every step waits for the step before it. On-time notes keep the whole chain moving.

CHECK YOURSELF

?

Which team member does the initial assessment visit?

Answer: A registered nurse (RN).

? What is the start of care?

Answer: It is the date of the first visit that begins care for the patient. The 5-day assessment deadline and the 60-day periods count from this date.

? Who watches visits as they happen?

Answer: The office team, on the Live Monitor. But team members in the field should not wait for the office. Call if you will be late.

3

Privacy and Security

● Everyone

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Explain what PHI is
- ✓ Protect your Alora account and your phone
- ✓ Know what you may and may not do with patient information
- ✓ Know exactly what to do if a phone is lost or an account is misused

Patients trust us with private information about their health and their homes. A federal law called HIPAA says we must protect it. This chapter shows you how, in simple steps.

WORDS TO KNOW

PHI

Protected Health Information. Any information that points to one patient and relates to health or care.

HIPAA

The federal law that protects patient health information.

Minimum necessary

Look at, use, and share only what you need to do your job.

Username and password

Your username says who you are. Your password proves it is you.

What is PHI?

PHI is any information that can identify a patient and is connected to health or care. If it can point to one patient, treat it as PHI. Here are examples.



Name, address, phone number, and birthday



Social Security number, Medicare number, Medicaid number



Diagnoses, medicines, vital signs, and test results



Visit notes, plans of care, and signatures



Photos and videos of the patient or the home



Phone messages and voice recordings about a patient

Your account is you

Your Alora account is your signature in the computer. Everything done with your account is treated as done by you. So you protect it.

FEDERAL RULE

HIPAA requires that each person who uses a health record system has their own unique login. It also requires Zenith to be able to check who opened or changed a record. **Act as if every tap is recorded.**

✓ DO

- ✓ Use a long password that only you know. A phrase of four or more unrelated words is strong and easy to remember.
- ✓ Lock your phone with a code, fingerprint, or face scan.
- ✓ Log out of Alora on any computer that other people use.
- ✓ Open only the patients you are assigned to, or that your job requires.
- ✓ Keep your phone with you. In a car, do not leave it where people can see it.
- ✓ If Alora asks for a second check, such as a code, follow it every time.

× DO NOT

- × Never share your username or password, even with a supervisor or a friend.
- × Never let another person clock in, write a note, or sign with your account.
- × Never look up a patient out of curiosity, such as a neighbor, a relative, or a public person.
- × Never take photos or screenshots of patient information with your personal camera, unless Zenith tells you to.
- × Never post, text, or chat about a patient on social media or in a personal messaging app. A missing name is not enough. Details can still identify a person.
- × Never leave patient screens open where family, visitors, or other people can read them.



GOOD PRACTICE ON PUBLIC WI-FI

When you are not sure a Wi-Fi network is safe, use your phone's mobile data instead. Alora keeps data protected, but a safe connection is better.

Patients, families, and visitors

The **patient** decides who may receive information about their care. A family member or friend is not allowed to see or hear patient information unless the patient says yes, or the law allows it.

? A neighbor or visitor asks you about the patient's health.

SAY THIS

"I cannot share information about patients. Please ask the patient or the family."

DO NOT

Do not confirm that the person is a patient, and do not share details. Even "yes, I visit that patient every Tuesday" is private information.

If something goes wrong

Mistakes happen. What matters is how fast you tell Zenith. Quick action can stop a small problem from becoming a large one. Report to your supervisor right away if:

- Your phone or tablet is lost or stolen.
- You think someone knows your password, or used your account.
- You sent patient information to the wrong person.
- You see someone looking at information they should not see.
- You opened a patient record by mistake.

1 Call your supervisor now

Use the phone number in Appendix A. Do not wait until tomorrow. Do not send a text.

2 Say what happened, and when

Give the facts: what, when, where, and which patients may be affected. Do not guess and do not hide anything.

3 Follow the instructions

The office may lock your account or change your password to keep patients safe. Do what the office tells you.



REPORT FAST AND TELL THE TRUTH

Hiding a problem is always worse than reporting it. A quick, honest report lets Zenith protect the patient and act on time. A late report can make the harm bigger.

☆ REMEMBER

- PHI is any health information that can identify a patient. Protect all of it.
- Your account is your signature. Never share it. Never use another person's.
- Lock your phone. Log out on shared computers. Keep screens private.
- Never post or message patient information in personal apps or on social media.
- If a device is lost or an account is misused, call your supervisor right away.

CHECK YOURSELF

? A co-worker says, 'Just clock me in, I am running late.' What do you do?

Answer: Say no. Clocking in for another person creates a false visit record. Tell your co-worker to call the office. If you are asked again, report it to your supervisor.

? You want to post a photo of a patient's garden with the caption 'my favorite patient'. Is this allowed?

Answer: No. A photo can identify a patient, and posting about a patient is not allowed. Keep patient information off social media.

? You find that your phone is missing after a visit. What is the first thing to do?

Answer: Call your supervisor right away, and tell them what happened. Do not wait to see if you find it.

? What does minimum necessary mean?

Answer: Look at, use, and share only the patient information you need to do your job.

4

Getting Started: Your Account and Your App

● Everyone

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Ask for your account and know what to receive
- ✓ Install the app and allow the right phone settings
- ✓ Log in for the first time and set your own password
- ✓ Do a practice run before your first patient

Before your first visit, you need three things: an account, the app on your phone, and a practice run. This chapter takes you through them one at a time. It takes about 20 minutes.

1



Get your account

The office creates it for you.



2



Set up your phone

Install the app. Allow location and notifications.



3



Practice

Do a practice visit with your supervisor.

What you need before you start

- ☐ A phone or tablet that can connect to the internet, with power and enough storage.
- ☐ A data plan or Wi-Fi to install the app.

- ☐ Your **username** and **temporary password** from the office.
- ☐ The name of the app your office wants you to use. See Appendix A.
- ☐ The phone numbers for the office and the on-call nurse, also in Appendix A.

Step 1. Get your account

Only the office can create your Alora account. They choose your **role**. The role decides what you see and what you can do. You will receive:

- A username (this is your name in Alora).
- A temporary password (you will change it).
- The name of the mobile app, and the web address for the computer version, if your job needs it.



DO NOT

Never use an account made for another person. Never ask for an account in someone else's name. If you did not receive your account, call the office.

Step 2. Install the app

1

Open the app store on your phone

On an iPhone, open **App Store**. On an Android phone, open **Google Play**.

2

Search for the app name

Type the exact name the office gave you, for example *Alora Anywhere* or *Alora Plus*. Several apps can have similar names. If you are not sure which one is right, ask the office before you install.

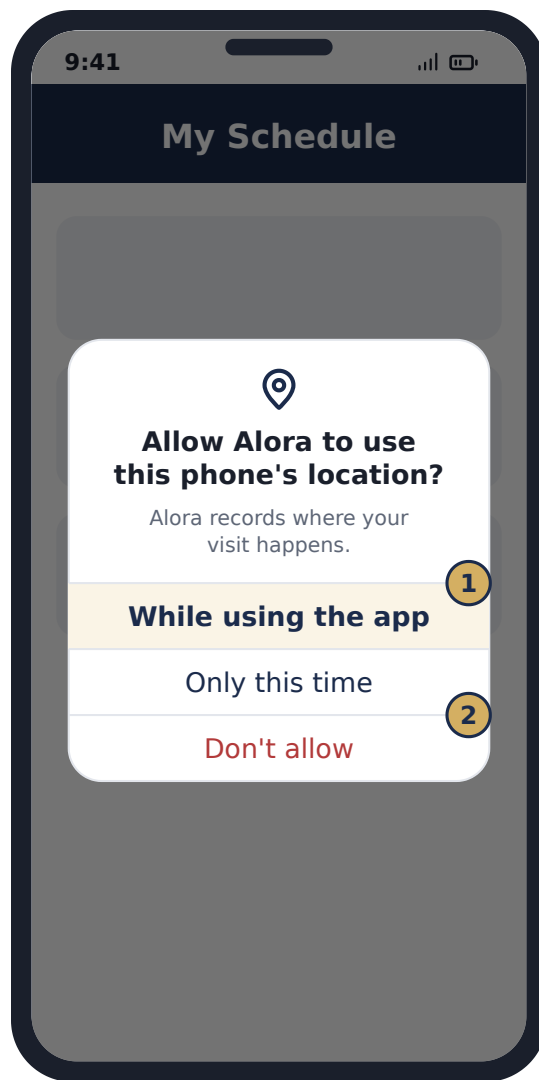
3

Tap Get or Install

Wait until the download finishes. Then tap **Open**.

Step 3. Allow the phone settings Alora needs

The first time you open the app, your phone asks for permission. These permissions make Alora work correctly. Read the screen and choose as shown below.



- 1 Choose this one.** Location is allowed while the app is open. This is what Alora needs to record where you clock in.
- 2 Do not choose this.** Without location, GPS clock in will fail. If you chose it by mistake, go to the phone Settings and turn location on for Alora.

The words may be different on iPhone and Android. Choose the option that allows location while the app is in use. If your phone shows a "Precise" switch, turn it on.

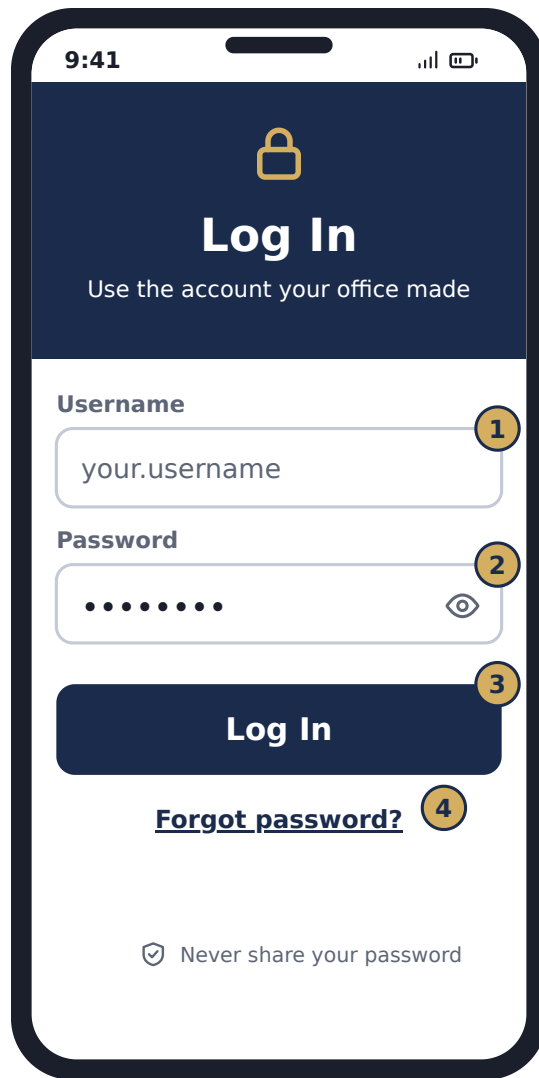
Also allow **notifications**. Alora uses them to tell you about schedule changes and open visits.



BATTERY SAVER

Battery saver modes can stop GPS and notifications. Turn battery saver off while you work. Charge your phone every night. Keep a car charger with you.

Step 4. Log in for the first time



- 1 Username.** Type the username your office gave you. Use small and capital letters exactly as given.
- 2 Password.** Type your password. Tap the eye to check what you typed.
- 3 Log In button.** Tap it once. Wait for the next screen.
- 4 Forgot password?** Tap this link to reset it, or call the office.

A simplified log in screen. Your screen may look a little different, but the four parts are the same.

1 Type your username and temporary password

Type them exactly as the office gave them. Watch the capital letters.

2

Tap Log In

Tap once and wait. The app may take a few seconds.

3

Choose your own password

Alora may ask you to make a new password. Choose a long password that only you know. Do not use a birthday, a phone number, or the word "password."

4

Look at your schedule

When you see a list of visits or a calendar, you are in.



LOGIN DOES NOT WORK?

If you cannot log in after two tries, stop. Call the office and ask for a password reset. Many systems lock an account after several wrong tries, and a locked account takes longer to fix.

Step 5. Do a practice run

Never make your first clock in on a real patient by yourself. Ask your supervisor for a practice run. If Zenith has a practice (training) patient in Alora, use it. If not, your supervisor will walk you through your first real visit.

During the practice run, do each of these:

- ☐ Open your schedule and find today's visit.
- ☐ Open the visit and find the address, the time, and the tasks.
- ☐ Find the Clock In button. Do not press it unless you are practicing.
- ☐ Find where to write a visit note.
- ☐ Find where the patient signs.
- ☐ Find the sync button or the sync notice.
- ☐ Find the help or contact information for the office.

If you use a computer

Office team members open Alora in a web browser. Use the web address in Appendix A. Log in with the same kind of username and password. Type the address yourself or use a saved bookmark. Do not follow links in emails or messages that ask you to log in, because criminals send false login pages.

Keep the app up to date

- Update only from the official app store.
- Update at home, before your shift. Do not update between visits.
- **Sync first, then update.** If the app has unsent work on the phone, sync it before you update.

☆ REMEMBER

- You need an account from the office, the right app, and a practice run.
- Allow location ("While using the app") and notifications.
- Choose a long password that only you know.
- If login fails twice, stop and call the office.
- Sync before you update the app.

CHECK YOURSELF

? Which location permission should you choose?

Answer: "While using the app" (or the nearest choice on your phone). It lets Alora record where you clock in. If you choose "Don't allow," GPS clock in will fail.

? Who creates your Alora account?

Answer: The office (an administrator). You never create an account for yourself, and you never use another person's account.

? Your login fails twice. What do you do?

Answer: Stop trying. Call the office for a password reset. More tries can lock the account.

? When should you update the app?

Answer: At home, before your shift, and only after you have synced all your work.

5

Finding Your Way Around Alora

● Everyone

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Name the main areas of Alora
- ✓ Find a patient or a visit with the search box
- ✓ Read the colors and status tags
- ✓ Use the three checks before you save anything

Alora can look large at first. But it is organized into a few areas, and each area has one job. When you know the areas, you always know where to go.

The office screen

Office team members use Alora on a computer. The picture shows the parts of the screen. Find each number in the list.

1 Menu. The main areas of Alora are listed here. Select one to open it.

2 Search. Type a patient or staff name to find the record fast.

3 Alerts (bell). A red dot means something new needs your attention.

4 Work area. The area you chose shows here: lists, forms, and boards.

5 Your name. Use it to see your account and to log out.

A simplified office screen. Menu names and places can differ in your Alora. The ideas are the same. Sample data. Not a real patient.

The main areas of Alora

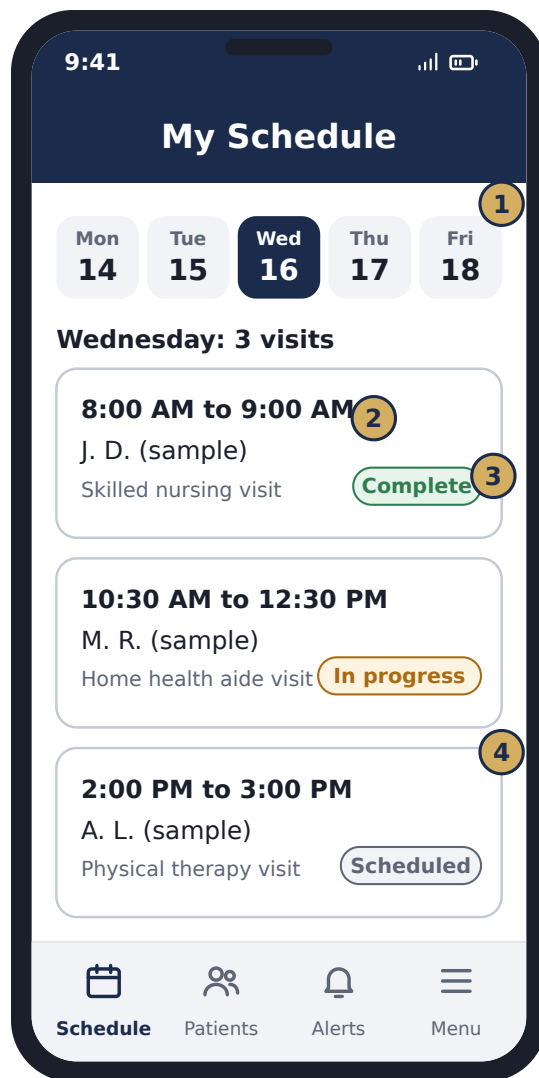
Area names can be different in your Alora. The jobs are the same.

AREA	WHAT YOU DO THERE	WHO USES IT
Dashboard	See what needs attention today.	Office team
Schedule	See, plan, and change visits.	Everyone (office plans; field staff see their own visits)

AREA	WHAT YOU DO THERE	WHO USES IT
Patients	Open a patient record: details, doctors, insurance, documents.	Everyone who is assigned or needs it
Clinical	Assessments (OASIS), plans of care, orders, visit notes.	Clinicians write. Office reviews.
Billing	Prepare, send, and follow claims.	Office team (billing)
Reports	Lists and totals that help you find problems.	Office team
Staff	Team member records, credentials, and expiration dates.	Office team (administration)
Messages and alerts	Notices about schedule changes, open visits, and tasks to finish.	Everyone

The phone screen

The mobile app is smaller and simpler. It has the areas that field staff use most. The main areas are usually at the bottom of the screen. On the schedule screen in the picture below, the four areas are **Schedule**, **Patients**, **Alerts**, and **Menu**.



- 1 **Day strip.** Tap a day to see the visits on that day. The dark box is the day you are looking at.
- 2 **Visit time.** This is the planned start and end. Plan your trip so you arrive on time.
- 3 **Status tag.** It shows where the visit is: Scheduled, In progress, or Complete.
- 4 **Visit card.** Tap a card to open the visit. The details, the tasks, and the Clock In button are inside.

Sample data. Not a real patient. Names are shortened on purpose. Keep patient names private.

Find a patient or a visit

Use the search box. Type part of a name. You do not need to type the whole name.

- 1 Type a few letters of the last name**
Search results appear as you type.
- 2 Check a second detail**
Two patients can have the same name, or a similar name. Check the birth date, the address, or another detail before you open a record.
- 3 Select the right patient**
Do not open a record you do not need. Every open is part of the record of who looked at the chart.

Read the colors, then read the words

Alora uses colors and words to show status. Colors help you scan quickly. But **always read the word too**. Colors can look different on different screens, and some people cannot tell certain colors apart.

Example of status colors (the same idea is used across Alora)

COLOR	MEANING	WHAT YOU DO
✓ Green	On time, or finished correctly.	Nothing. Keep going.
⌚ Yellow or orange	Late, or needs attention soon.	Act now, before it becomes red.
⚠ Red	A problem. A missed visit or a missing record.	Act immediately. Tell your supervisor.
✓ Gray	Finished or not active.	Nothing, unless a record needs a correction.

Alerts and messages

Alora can send you notifications. It can tell you about schedule updates and open visits that need a team member. Turn notifications on, as shown in Chapter 4.



DO NOT WAIT FOR AN ALERT

Alerts help, but they are not perfect. Phones can be silent, the internet can be slow, and settings can be different. Check your schedule in the morning and in the evening, and before you leave home for each visit. Office team members should look at the Live Monitor often, and should not depend only on alerts.

ZENITH STANDARD

During your working hours, answer messages and calls from the office within **2 hours**.

The three checks before you save

The most common mistakes in health records are simple. The note goes to the wrong patient. The date is wrong. The wrong visit is open. You can prevent nearly all of these with three quick checks.

1



Right patient?

Check the name and one more detail.



2



Right date?

Check the date and time at the top.



3



Right visit?

Check the service, such as nurse or aide.

☆ REMEMBER

- Alora has a few main areas. Each area has one job.
- Use the search box. Check a second detail before you open a patient.
- Colors help, but always read the word. Green is fine, yellow needs attention, red is a problem.
- Do not wait for an alert. Check your schedule morning and evening.
- Before you save: right patient, right date, right visit.

CHECK YOURSELF

? Why do you check a second detail before opening a patient?

Answer: Two patients can have the same or similar names. A second detail, such as the birth date, stops you from opening or changing the wrong record.

? A visit is shown in red. What does it mean, and what do you do?

Answer: Red means a problem, such as a missed visit or a missing record. Act right away and tell your supervisor. Also read the word on the screen to know the exact problem.

? What are the three checks before you save?

Answer: Right patient, right date, and right visit.

PART B

In the Field

These eight chapters are for clinicians and aides who visit patients. They follow the visit from the schedule to the last sync. Office team members should read chapters 6 to 11 too. You support the people in the field, and you need to know what their screens look like.

6

Your Schedule

● Clinicians

◆ Aides

About 5 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Open your schedule and read a visit before you go
- ✓ Pick up an open visit the right way
- ✓ Know exactly what to do if you cannot make a visit
- ✓ Plan your day so you arrive on time

Your schedule is the list of visits planned for you. It is the most important screen in the app. Patients wait for you, so you need to know where to go and when.

WORDS TO KNOW

Schedule

The plan of visits: who goes, to which patient, and when.

Visit card

One box on the schedule. It stands for one visit.

Open visit

A visit that has no team member yet. Alora can tell team members about it.

Scheduler

The office team member who plans visits.

Open your schedule

You saw the schedule screen in Chapter 5. Here are the steps.

- 1 Open the app**
Log in if the app asks you to.
- 2 Choose Schedule**
It is usually the first area at the bottom of the screen.
- 3 Pick the day**
Select a day in the day strip. Today is usually marked.
- 4 Open a visit card**
Select a card to see everything about that visit.

What to read before every visit

A visit card holds the facts you need. Read all of them before you leave for the visit.

 The patient and the service you are giving	 The date and time window	 The address . Open the map before you leave.
 The tasks you must do, from the plan of care	 Any safety notes about the home, such as a pet, steps, or a parking problem	 Who to call if you have a problem

Check your schedule three times a day

Plans change. A visit can be moved, added, or cancelled after you plan your day. Checking three times keeps you from being surprised.

- 1 The evening before**
Look at tomorrow. Plan your trips and supplies.



2**The morning of**

Look for changes. Check your phone charge.

**3****Before each visit**

Read the visit card again before you drive.

Open visits

Sometimes a visit has no team member. Alora can send a notice to team members who may be able to take it. If you want it, follow the notice.

1**Read the notice**

Check the day, time, place, and service.

2**Ask yourself three questions**

Do I have the license or certification for this service? Am I free at this time? Can I get there on time?

3**Accept only if all three answers are yes**

Follow the instructions in the notice. In some cases you accept in the app. In some cases you call the scheduler. The office will tell you which.

4**Check that the visit is on your schedule**

If the visit does not appear, call the scheduler. Do not go to a visit that is not on your schedule.

If you cannot make a visit

Life happens. You may be sick, stuck in traffic, or have an emergency. The patient still needs care, so the office needs time to find another team member.

❓ Can you still arrive close to the planned time?

YES, A LITTLE LATE

Go to the visit. Call the office if you will be more than **15 minutes** late. Clock in at the real time you arrive.

NO, OR YOU CANNOT GO

Call the office now. Do not wait. Say the patient's name, the visit time, and why you cannot go. The office will find another team member.

ZENITH STANDARD

Tell the office about a visit you cannot make: **as soon as you know, and before the visit starts when possible.**

✓ **DO**

- ✓ Call the office, even if you think it is late. A phone call is fast and clear.
- ✓ Give the patient's name and the visit time.
- ✓ Write down who you spoke to and when.

✗ **DO NOT**

- ✗ Do not send only a text or a message and wait for a reply.
- ✗ Do not swap visits with another team member on your own.
- ✗ Do not call the patient to cancel. The office does that.

Back-to-back visits

- Plan travel time between patients. Leave early.
- Finish one visit completely before you start the next. Clock out of one patient before you clock in to the next.
- Never clock in for a visit while you are still at another patient's home.



DRIVE SAFELY

Never use your phone while you drive. Park first, then read or call. Patients are waiting for you, but your safety and the safety of other people come first.

☆ **REMEMBER**

- Read every visit card before you go: patient, time, address, tasks, and safety notes.
- Check your schedule the evening before, the morning of, and before each visit.
- Accept an open visit only if you have the license, the time, and a way to arrive on time.
- If you cannot make a visit, call the office right away. Do not send only a text.
- Clock out of one patient before you clock in to the next.

CHECK YOURSELF

? **When should you check your schedule?**

Answer: Three times: the evening before, the morning of the visit, and before each visit.

? **You are sick this morning and have two visits. What do you do?**

Answer: Call the office right away and tell them the patient names and visit times. Do not wait and do not only send a text. The office will find other team members.

? **May you trade a visit with a friend who also works for Zenith?**

Answer: No. Only the office can change who goes to a visit. The patient's care, the records, and the billing all depend on the schedule being correct.

7

The Visit Day, Step by Step

● Clinicians

◆ Aides

About 5 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Follow all ten steps of a visit in the right order
- ✓ Know what to do before, during, and after every visit
- ✓ Handle the common surprises calmly
- ✓ Finish each day with the end-of-day check

Every visit follows the same pattern. When you learn the pattern once, you can use it for every patient, every day. This chapter is the map. The next chapters explain the hardest steps in detail.

The pattern in one look



Before

1. Check your schedule
2. Prepare your phone and supplies



At the home

3. Arrive and greet
4. Clock in
5. Give care
6. Write the note

7. Get signatures

8. Clock out



After

9. Sync your work

10. Leave safely and check the next visit

Step by step

Before the visit

1 Check your schedule
Read the visit card. Know the patient, the time, the address, the tasks, and any safety notes. See Chapter 6.

2 Prepare your phone and supplies
Charge your phone. Turn battery saver off. Bring your ID badge, your supplies, and a charger. Plan the trip and leave early.

- ☐ Phone is charged (more than half).
- ☐ Location is on. Notifications are on.
- ☐ ID badge and supplies are in your bag.
- ☐ You know the address and how to get there.

At the patient's home

3 Arrive and greet
Park safely. Put your ID badge where it can be seen. Knock, say your name, and say you are from Zenith. Wash your hands or use hand sanitizer.

4 Clock in

Wait until you are inside and ready to begin care. Check the visit, wait for "Location found," and tap **Clock In**. See Chapter 8.

5

Give care

Follow the plan of care and the task list. Ask the patient what they want and explain what you are doing. Respect the patient's choices. If a task is not done, remember the reason so you can write it.

6

Write the note

Record your vital signs, your tasks, and what you saw. Do it while the visit is fresh in your mind. See Chapter 10.

7

Get signatures

After care is done, ask the patient to sign on the screen. Then sign yourself. See Chapter 11.

8

Clock out

Tap **Clock Out** at the home, when care is finished. Wait for the message that confirms it. See Chapter 8.

After the visit

9

Sync your work

Make sure everything you did is sent to Alora. If you have no internet, sync later, but **before you finish work for the day**. See Chapter 9.

10

Leave safely and check the next visit

Say goodbye to the patient. Make sure the door is closed as the patient wants. Then open your schedule and read the next visit before you drive.

When the visit does not go as planned

Surprises happen. Here is what to do for the most common ones. Chapter 20 has more.

? The patient does not answer the door.

DO THIS

1. Knock again. Call the patient's phone.
2. Call the office and tell them. Wait for instructions.
3. Do not clock in. No care was given.

EMERGENCY

If you think the patient may be hurt, sick, or in danger, **call 911 first**. Then call the office.

WHAT HAPPENS	WHAT YOU DO
The patient refuses care or a task.	Respect the patient's choice. Explain why the care matters, once. Do not argue. Write what was refused and what you said. Tell your supervisor.
The family asks you to do something that is not on the plan.	Politely say you can only do the tasks on the plan. Call your supervisor if the family still asks. Do not do extra tasks without approval.
The patient is sicker than usual.	Do not wait. Follow your training. Call your supervisor or the on-call nurse. Call 911 for an emergency. Write what you saw.
The home is unsafe (for example, a threat, a weapon, or violence).	Leave. Call 911 if there is danger. Then call the office. Your safety comes first.
The patient asks you to leave early.	Respect the request. Clock out at the real time. Write what happened. Tell your supervisor.
You cannot finish the visit (for example, you feel ill or there is an emergency).	Call the office. Clock out at the real time. Write what was done and what was not.

The end-of-day check

Before you finish work, check each item. It takes two minutes. It prevents most problems the next morning.

- ☐ Every visit today has a clock in and a clock out.
- ☐ Every visit note is written and saved.
- ☐ Signatures are collected, or the reason is written.
- ☐ Every task that was not done has a reason written.
- ☐ Everything is synced. The waiting list is empty.
- ☐ Any problem, late visit, or change in a patient was reported to the office.
- ☐ Tomorrow's schedule has been checked.
- ☐ Your phone is charging.

☆ REMEMBER

- Every visit: check, prepare, arrive, clock in, care, note, signature, clock out, sync.
- Clock in when you are at the home and ready. Clock out when care is finished, still at the home.
- If the patient does not answer and you fear danger, call 911 first.
- Do the end-of-day check before you stop working.

CHECK YOURSELF

? In which order do you do these: write note, clock in, clock out, sync, give care?

Answer: Clock in, give care, write note, clock out, sync.

? The patient does not answer the door and you are worried. What do you do first?

Answer: If you think the patient may be in danger, call 911 first, then call the office. If there is no sign of danger, call the patient, then the office, and wait for instructions. Do not clock in.

? When should you sync?

Answer: As soon as you have internet, and always before you finish work for the day.

8

Clocking In and Out (EVV)

● Clinicians

◆ Aides

About 10 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Explain what EVV records and why it matters
- ✓ Clock in and clock out correctly with GPS
- ✓ Use the telephone backup when the app cannot be used
- ✓ Fix a forgotten clock out the honest way

Clocking in and out is the most important action you do in Alora. It proves that a visit happened. It must be true, every time. This chapter explains how to do it well.

WORDS TO KNOW

EVV

Electronic Visit Verification. Proof that a visit happened.

GPS

A phone feature that finds where the phone is.

Telephone method

A backup way to clock in and out by calling a number. It is used when the app cannot be used.

Exception

A visit record that does not match the plan, such as a missing clock out.

What EVV records

EVV records six facts about every visit. You give most of them when you clock in and out.



1. The service

What kind of visit:
nurse, aide, therapy



2. The patient

Who received the
care



3. The team member

Who gave the care.
This is you.



4. The date

The day of the visit



5. The place

Where the visit
happened



6. The times

When care started
and ended

FEDERAL RULE

Federal law (the 21st Century Cures Act) requires EVV for Medicaid home health and personal care visits. These are the facts it asks for. The office decides how Zenith's EVV records reach the state system. You do not need to do anything different. You only need to clock in and out correctly.

Follow the same clock in and clock out steps for every visit, unless the office tells you something different.

Two ways to clock in

Alora offers two methods. The app with GPS is the main method. The telephone method is the backup.

? Can you use the Alora app on your phone right now?

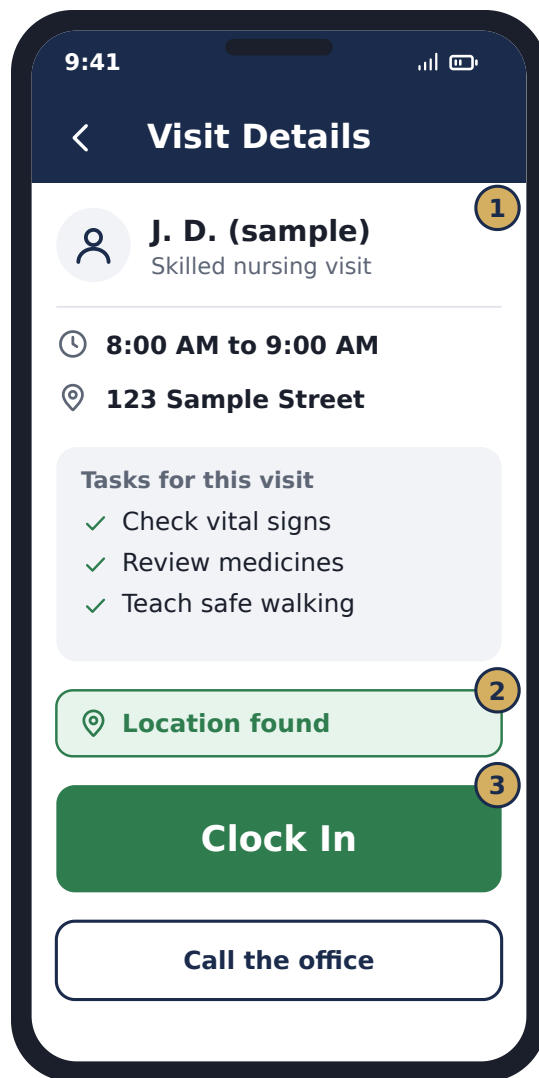
YES

Use the app with GPS. Tap **Clock In** and **Clock Out**. The app records the time and the place. It can do this even with no internet.

NO

Use the telephone method. This is for times when the phone is broken, the app will not open, or you have no smartphone. Call the EVV number from Appendix A. Then tell the office you used it.

Clock in with the app



- 1 Check the visit.** Make sure the patient, the service, and the time are the ones you expect.
- 2 Location found.** A green tag means your phone knows where you are. If you do not see it, wait a few seconds. Do not clock in until it appears.
- 3 Clock In.** Tap it once when you are inside the patient's home and ready to start care.

Sample data. Not a real patient.

1

Go inside the home

Clock in only when you are at the patient's home and ready to start care. Do not clock in in the car or on the street.

2

Open the visit



Open the visit card from your schedule. Check the patient, the service, and the time.

3

Wait for "Location found"

A green tag means your phone knows where you are. If it does not appear in a few seconds, read the problem table below.

4

Tap Clock In once

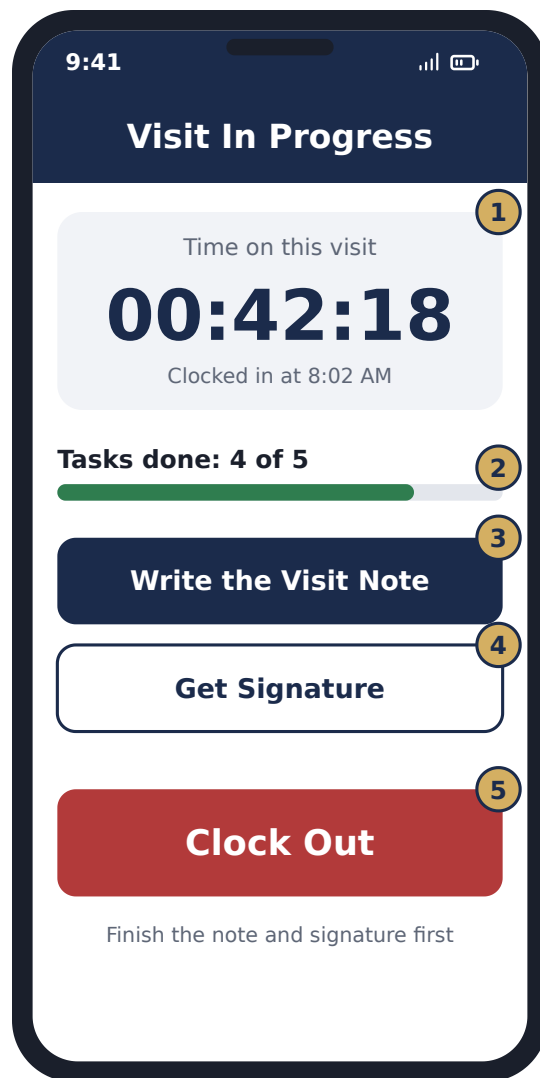
Do not tap many times. Wait for the screen to change.

5

Check that it worked

The screen should say the visit is **In progress**, and a timer should run. If not, call the office.

Clock out with the app



- 1 **Timer.** It counts the time since you clocked in. You do not need to watch it.
- 2 **Tasks done.** The bar shows how many planned tasks you finished.
- 3 **Visit note.** Write what you did and what you saw. The next chapter explains how.
- 4 **Signature.** Ask the patient to sign when signatures are needed.
- 5 **Clock Out.** Tap it last, when care is finished and you are still at the home.

Sample data. Not a real patient.

- 1 **Finish care first**
Finish your tasks. Write your visit note. Collect the signatures.
- 2 **Stay at the home**

Clock out while you are still at the patient's home. Do not clock out later from another place.

3

Tap Clock Out once

Wait for the confirmation message.

4

Check that the visit shows Complete

Open your schedule. The visit should show **Complete**. If it does not, something is missing. Fix it now.

The telephone backup

Use the telephone method only when the app cannot be used. The office will train you. In general it works like this:

1

Call the EVV number

The number is in Appendix A. The office will tell you which phone to call from.

2

Follow the voice instructions

You may be asked for your ID and the patient's ID or the visit number. Listen and answer clearly.

3

Call again when care ends

Use the same number to clock out.

4

Tell the office

Tell the office you used the telephone method, so they can check that the visit is correct.

The rules of clocking in and out

✓ **DO**

- ✓ Clock in when you are at the home and ready to start care.
- ✓ Clock out when care ends, while still at the home.
- ✓ Use the real time. If you are late, clock in late.
- ✓ Do one visit at a time.
- ✓ Tell the office when something goes wrong, the same day.

✗ **DO NOT**

- ✗ Do not clock in before you arrive, or from the car.
- ✗ Do not clock in at the planned time if you arrived later.
- ✗ Do not clock in for another person, or let another person clock in for you.
- ✗ Do not clock in and then leave.
- ✗ Do not change or ask to change times to match the schedule.



WHY THIS MATTERS SO MUCH

Clock times are legal records. They are used to bill Medicaid and other payers. A false time, even a small one, can be treated as fraud. It can lead to loss of a license and to penalties for the person and for Zenith. The truth is always the safest choice.

What is an exception?

An **exception** is a visit record that does not match what was planned. For example, you forgot to clock out, or you clocked in far from the patient's address. Alora flags these visits. The office reviews them before the visit is finished and sent onward.

1



Something is off

For example, no clock out.



2



Alora flags it

The visit goes to a review list.



3



Office asks you

You explain what really happened.



4



Office fixes it

The office records the truth, with a reason.

An exception is not a punishment. It is a check. If you explain it honestly and quickly, it is fixed easily.

ZENITH STANDARD

The office reviews EVV exceptions within **1 business day**. Answer the office quickly when they ask you about one.

You forgot to clock out

1

Do not guess the time

Do not tap Clock Out now if you already left the home. A late clock out creates a false time.

2

Call the office as soon as you remember

Tell them which patient and visit.

3

Tell the truth about the real end time

Say about when you really finished. If you are not sure, say so. The office can check your note and your phone.

4

Let the office correct it

The office records the correct time and writes the reason. You may be asked to add a short note.

Problem table

PROBLEM	WHAT TO DO
"Location not found" or no green tag	Check that location is on for Alora. Move near a window or a door. Wait 30 seconds. Try again. If it still fails, call the office and use the telephone method.
The app will not open	Close it and open it again. Restart the phone. If it still fails, use the telephone method and tell the office.
There is no internet or signal	Clock in anyway. The app saves your clock in on the phone and sends it later. See Chapter 9.
You clocked in at the wrong patient	Stop. Do not continue. Call the office right away. They will fix it and record the reason.
You forgot to clock in	Do not clock in late and pretend. Call the office and tell them the real start time.
Two patients live in the same home	Ask your supervisor how to clock in. The schedule should show one visit for each patient.
Your phone died during the visit	Charge it. Use the telephone method if needed. Tell the office the real times.
The patient sends you away early	Clock out at the real time. Write the reason in the note. Tell your supervisor.

☆ REMEMBER

- EVV records the service, the patient, the team member, the date, the place, and the times.
- Clock in at the home when care begins. Clock out at the home when care ends.
- Use the app with GPS. Use the telephone backup only when the app cannot be used.
- Never clock in or out for another person. Never change times to match the schedule.
- If you forgot to clock out, call the office and tell the truth about the real time.

CHECK YOURSELF

? You arrive at the patient's home at 10:12, but the visit was planned for 10:00. What time do you clock in?

Answer: 10:12, the real time. Always use the real time. If you will be more than **15 minutes** late, also call the office.

? You finished the visit and drove away. Then you remember you did not clock out. What do you do?

Answer: Call the office. Do not tap Clock Out now, because the time would be false. Tell the office about when you really finished. They will correct the record and write the reason.

? May you clock in while you are still in your car outside the home?

Answer: No. Clock in only when you are at the home and ready to begin care.

? What is an exception?

Answer: A visit record that does not match the plan, for example a missing clock out. Alora flags it, and the office reviews it.

9

Working Without Internet: Offline Mode and Sync

● Clinicians

◆ Aides

About 5 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Explain what offline mode does
- ✓ Keep working when there is no signal
- ✓ Sync correctly and check that nothing is left behind
- ✓ Avoid the mistakes that can lose work

Some homes have no signal. Some roads have no signal. Alora is built for this. The app keeps your work on the phone and sends it when the internet comes back. This chapter shows you how to do it safely.

WORDS TO KNOW

Offline

Not connected to the internet.

Sync

Short for synchronize. The phone sends its saved work to Alora, and gets new information back.

Waiting list

Items saved on the phone that are not yet sent.

What happens offline

The Alora mobile app can capture your clock in and out with GPS, your clinical documentation, and your signatures **without internet**. It saves everything on the

phone. When the phone connects again, the app sends the saved items to Alora.

1



No internet

Keep working. The app saves your work on the phone.



2



Internet returns

The app sends the saved work. You may tap Sync Now.



3



The office sees it

Alora now has your visit, your note, and your signature.

What works and what waits

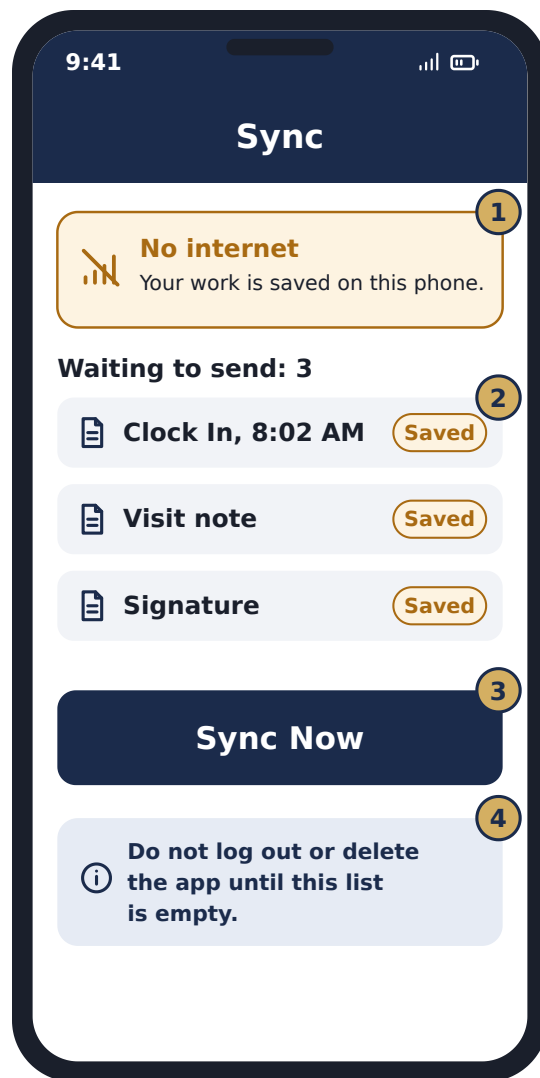
WORKS WITHOUT INTERNET	NEEDS INTERNET
YES Clock in and clock out with GPS	WAITS New schedule changes and notices
YES Writing visit notes and clinical forms	WAITS Open visit notices
YES Patient and team member signatures	WAITS The office seeing your work (until you sync)



TIP

Until you sync, the office may not see your clock in yet. If you work in a place with no signal, tell the office, so they do not worry that you are late.

How to sync



- 1 Offline notice.** The phone has no internet. Alora keeps working and saves your work on the phone.
- 2 Waiting list.** These items are saved on the phone but not yet in the office system.
- 3 Sync Now.** Tap it when the phone has internet again. The list should become empty.
- 4 Warning.** Do not log out or delete the app while items are waiting. You could lose them.

A simplified sync screen. Your app may sync by itself when internet returns, and may show the waiting items in a different place.

1

Look for the notice

The app shows that you are offline, and how many items are waiting.

2

Find internet

Use Wi-Fi or a place with mobile signal. Stand near a window. Do this at the end of the visit, or on the way to the next one, when you are parked.

3

Sync

Some apps sync by themselves. If yours has a **Sync Now** button, tap it once. Wait.

4

Check that the list is empty

When the waiting list shows zero items, everything is sent. If items are still waiting, wait a minute and try again.



PROTECT YOUR UNSENT WORK

While items are waiting on the phone, **do not log out, do not delete the app, do not reset the phone, and do not change phones.** You could lose the work that is not yet sent. If you must replace a phone, call the office first.

If sync does not finish

PROBLEM	WHAT TO DO
Items stay on the list	Move to a place with better signal. Wait a minute. Tap sync again.
An error message appears	Write down the words on the screen. Do not delete the app. Call the office.
The app closes by itself	Open it again. Check the waiting list. Do not log out.
You cannot sync before the end of the day	Call the office and say so. Keep the phone charged and safe. Sync as soon as you can.

ZENITH STANDARD

Sync all your work **before you finish work for the day.**

Good habits for offline work

- ☐ Keep your phone's date and time on **automatic**. Correct time keeps your records correct.
- ☐ Keep enough storage free on the phone. Delete old photos, not Alora data.
- ☐ Keep the phone charged. A dead phone cannot sync.
- ☐ Keep the app updated, but only after syncing.
- ☐ Do not switch on airplane mode during a visit unless the office asks.

☆ REMEMBER

- Alora works offline: clock in and out, notes, and signatures are saved on the phone.
- Sync when the internet returns. The waiting list should be empty.
- Never log out or delete the app while items are waiting.
- Tell the office if you work where there is no signal.
- Sync **before you finish work for the day.**

CHECK YOURSELF

? **You are in a home with no signal. Can you still clock in?**

Answer: Yes. The app can record your clock in with GPS without internet, and it sends it later when you sync.

? **Three items are waiting on the list. Your phone is slow, so you want to log out and restart. Is that a good idea?**

Answer: No. Do not log out while items are waiting. Try to sync first. Restart the phone only if the office tells you to.

? **How do you know that sync is finished?**

Answer: The waiting list shows no items, and the app does not show an error.

10

Visit Notes and Good Documentation

● Clinicians

◆ Aides

About 9 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Write a visit note that is true, complete, clear, and on time
- ✓ Tell a fact from an opinion
- ✓ Correct a mistake the right way
- ✓ Know what never belongs in a note

A visit note is the written story of one visit. Another team member may read it tomorrow. A reviewer may read it next week. An auditor may read it years from now. It must make sense to someone who was not there.

WORDS TO KNOW

Visit note

The written record of one visit.

Documentation

All the written or recorded information about care.

Vital signs

Basic body measurements, such as blood pressure, pulse, temperature, and oxygen level.

Addendum

A new entry that adds to, or corrects, an earlier note. The first note stays.

The five qualities of a good note

1



True

Only facts you saw, did, or heard.



2



Complete

Every task, number, and event is there.



3



Clear

Plain words. Someone else can understand it.



4



On time

Written the same day as the visit.



5



Yours

Written in your own account, about your visit.

ZENITH STANDARD

Write and save your visit notes by this time: **the end of the same day.**

Write a note step by step

9:41

< Visit Note

Vital signs

BP	Pulse	Temp	O2
128/76	72	98.2	97%

Tasks

- ✓ Checked blood pressure
- ✓ Reviewed medicines
- ⚠ Wound care: not done

Notes

Walked with walker. No pain.
Skin clean and dry. Lungs clear.
Wound care not done: patient refused. Called my supervisor at 8:40 AM. Patient was calm.

Save Note

- 1 **Vital signs.** Type the numbers you measured. Type only real measurements.
- 2 **Tasks.** Each planned task shows done or not done. A task that was not done needs a reason.
- 3 **Notes.** Write facts: what you did, what you saw, what the patient said, and who you called.
- 4 **Save Note.** Tap it. Wait for the message that says the note is saved.

Sample data. Not a real patient. Notice that the reason for the missed task is written in the note.

1 Open the visit and check it

Right patient, right date, right visit. You did this in Chapter 5.

2

Enter vital signs

Type the numbers you measured. Type only real measurements. If a number looks strange, measure it again. If it is outside the range in the plan of care, tell the nurse right away.

3

Mark each task

Mark a task **done** only if you did it. If it was **not done**, write why.

4

Write the notes

Write what you did, what you saw, what the patient said, and who you called. See below for how to write this well.

5

Read it once

Check the numbers and the spelling. Ask: could a stranger understand this?

6

Save

Tap **Save Note**. Wait for the message that the note was saved.

Facts, not opinions

A fact is something you can see, measure, or hear. An opinion is what you think it means. Write facts. You may write your opinion, but mark it clearly and explain the facts that led to it.

OPINION (AVOID)	FACT (WRITE THIS)
Patient seems angry.	Patient raised voice and said, "I do not want a bath today."
Patient ate poorly.	Patient ate about one quarter of lunch (half a sandwich, no soup).
Patient is weak.	Patient needed help from two people to stand. Patient sat down after 3 steps.
Wound looks bad.	Wound on left heel is 3 cm long, red around the edge, with yellow fluid on the bandage.

OPINION (AVOID)	FACT (WRITE THIS)
Family is rude.	Son asked me to leave the room three times. I left and called my supervisor.

A good note and a poor note

✗ POOR NOTE

"Visit done. Patient fine. Nothing new."

Why it is poor: it says nothing. A reader cannot tell what was done, what was seen, or whether the patient was really fine.

✓ GOOD NOTE

"Arrived 8:02. BP 128/76, pulse 72, temp 98.2, O2 97%. Helped with bath and dressing. Walked with walker to the kitchen, steady, no pain reported. Skin clean and dry. Ate half of breakfast. Patient said, 'I feel tired today.' Told my supervisor at 8:40."

Why it is good: it has numbers, actions, observations, the patient's own words, and who was called.

✗ POOR NOTE

"Patient refused wound care."

Why it is poor: no reason, no time, and no action. A reviewer will ask many questions.

✓ GOOD NOTE

"Wound care not done. At 8:30 patient refused. I explained that the wound must stay clean. Patient still said no. I did not force it. I called my supervisor at 8:40. A nurse will visit this afternoon."

Why it is good: it says what happened, why, what you said, and what you did next.

Always explain what was not done

If a task on the plan was not done, the note must say **why** and **who you told**. Examples of reasons: the patient refused, the patient was sick, the equipment was not available, or the patient was not home. Never leave a task empty and never mark it done if it was not done.

Report changes right away

The note is not the place to report an emergency. If you see a change in the patient, call. For example: new pain, a fall, trouble breathing, confusion, a new wound, a fever, or a very high or low measurement.

FEDERAL RULE

The agency must promptly alert the doctor to any change in the patient's condition or needs that suggests the plan of care should be changed. (42 CFR 484.60) Your call to the nurse or supervisor starts this process. Aides call the nurse. Nurses call the doctor.

What never belongs in a note

✓ DO

- ✓ Use the patient's own words, with quotation marks, when they matter.
- ✓ Use only abbreviations that Zenith approves. When in doubt, write the whole word.
- ✓ Write times as you know them, such as "8:40".
- ✓ Write about **this visit only**.

✗ DO NOT

- ✗ Do not copy and paste an old note. Every visit needs its own note.
- ✗ Do not write a note before the visit, and do not write what you plan to do.
- ✗ Do not write opinions about the family or other patients. Never use another patient's name.
- ✗ Do not use vague words such as "fine", "normal", or "ok" alone.
- ✗ Do not write anything you did not see or do.

Fix a mistake the right way

Everyone makes mistakes. The right way to fix a mistake keeps the first entry and adds a clear correction. Never hide the mistake.

1

Do not erase or overwrite

Do not try to make the old entry disappear, and do not change the date.

2

Add an addendum or a late entry

Use the correction or addendum feature in Alora, if it has one. If not, ask your supervisor how to do it.

3

Say what was wrong and what is right

Example: "Correction: BP entered as 182/76. Correct reading is 128/76."

4

Let the system show the real date and time

The correction must show when it was really made, and who made it.



LATE NOTE?

If you forgot to write a note, write it now, and label it as a **late entry**, with today's real date and time. Do not make it look like it was written on the visit day. Tell your supervisor why it was late.

What happens after you save

Your note goes into the patient's record. The clinical reviewer may read it. If something is missing or unclear, the reviewer may ask you a question. Answer it quickly and honestly, with an addendum if needed. This is normal and helps keep records correct.

☆ REMEMBER

- A good note is true, complete, clear, on time, and yours.
- Write facts. Quote the patient. Give numbers.
- If a task was not done, write why, and who you told.
- Never copy an old note. Never write before the visit.
- Fix a mistake with an addendum. Never erase or backdate.

CHECK YOURSELF

? Which is a fact: 'Patient is lazy' or 'Patient stayed in bed and said, I am too tired to get up'?

Answer: The second one. It says what you saw and heard. The first is an opinion.

? You wrote the wrong blood pressure in a saved note. How do you fix it?

Answer: Add a correction or addendum. Say what was wrong and what is right. Do not erase the first entry and do not change the date.

? It is three days later and you forgot to write a note. What do you do?

Answer: Write it now and mark it as a late entry with today's real date. Tell your supervisor why it was late. Never make it look like it was written on the visit day.

? May you copy yesterday's note, because the visit was the same?

Answer: No. Each visit needs its own true note. A copied note can be false. Copied notes are also a known risk in audits.

11

Signatures and Consent

● Clinicians

◆ Aides

About 5 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Collect a patient signature the right way
- ✓ Know who may sign, and who may never sign
- ✓ Handle a patient who cannot or will not sign
- ✓ Make sure the patient understands what they sign

A signature shows that the patient agrees, or that care was given. It is a legal act. This chapter shows how to collect signatures honestly, in a way that respects the patient.

WORDS TO KNOW

Consent

The patient says yes after understanding what will happen.

Representative

A person the law allows to act for the patient, for example a health care surrogate or a person with power of attorney.

Electronic signature

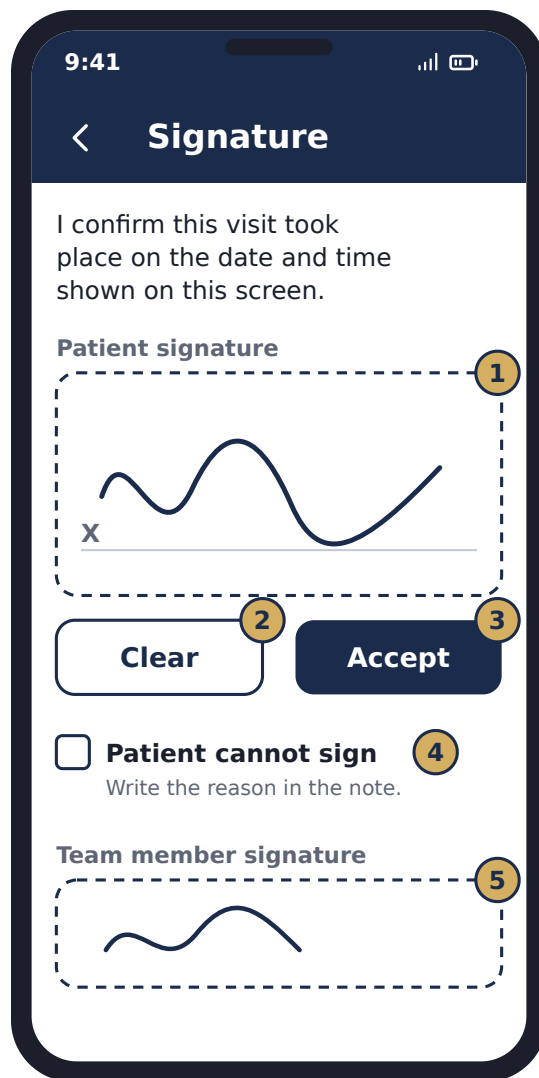
A signature made on a screen with a finger. It counts as a real signature.

Why we collect signatures

Signatures do two things. They show that the patient received the care at the visit. They also show that the patient agreed to the care. Some payers and programs require a

patient signature for each visit. Zenith may also require it. Ask the office which visits need a signature.

Collect a signature step by step



- 1 **Patient signature box.** The patient signs with a finger. Hand the phone to the patient.
- 2 **Clear.** Wipes the box so the patient can try again.
- 3 **Accept.** Saves the signature. You cannot sign for the patient.
- 4 **Patient cannot sign.** Use this only when the patient truly cannot sign. Write the reason in the visit note.
- 5 **Team member signature.** You sign too, to confirm the visit.

Sample data. Not a real patient.

1 Finish care first

Ask for the signature at the end of the visit, after care is done. Never ask before care is given.

2

Explain what they are signing

Say it in simple words: "This shows that I visited you today and gave you care."

3

Hand the phone to the patient

The patient signs in the box with a finger. Give them time. Help them hold the phone steady if they need it, but do not guide their hand.

4

Check the signature, then accept

If it is not clear, tap **Clear** and ask the patient to sign again. Then tap **Accept**.

5

Sign yourself

Your signature confirms that you gave the care and that the record is true. Sign only after you finish.

Who may sign

SITUATION	WHAT TO DO
The patient can sign.	The patient signs.
The patient cannot sign, and has a representative who is legally allowed to act for them.	The representative signs. Write their name and their relationship to the patient. Ask the office if you are not sure who may sign.
The patient cannot sign, and there is no representative.	Use the option for patient cannot sign. Write the reason in the visit note. Tell your supervisor the same day. Follow the instructions you get.
The patient refuses to sign.	Do not pressure. Respect the choice. Write that the patient refused, and why if the patient says. Tell your supervisor.

✓ **DO**

- ✓ Get the signature after care.
- ✓ Give the patient time to read and to ask questions.
- ✓ Use an interpreter or a translated form if the patient does not understand English.
- ✓ Write the reason in the note if there is no signature.

✗ **DO NOT**

- ✗ Never sign for a patient, or draw the patient's signature.
- ✗ Never ask another person to sign for the patient unless they are the legal representative.
- ✗ Never get a signature before care, or collect signatures for future visits.
- ✗ Never ask a patient to sign something they cannot read or do not understand.

Understanding matters

FEDERAL RULE

Patients have the right to be informed of their rights and of the care in a language and manner they understand. (42 CFR 484.50)

If the patient speaks another language, or has trouble hearing, reading, or seeing, a signature does not mean consent unless the patient understands. Use an interpreter, read the words aloud, or use a form in the patient's language. Tell your supervisor if you need help.

Consent at the start of care

The registered nurse gives the patient important papers at the first visit, such as the notice of patient rights. The nurse explains them and collects signatures. If you are an RN, this is part of your first visit. If you are an aide or another discipline, you may be asked to check that these papers exist before you start. If you are not sure they were done, call the office.

☆ **REMEMBER**

- Ask for the signature after care, never before.
- The patient signs, or a legal representative signs. You never sign for a patient.
- If a patient cannot or will not sign, write the reason and tell your supervisor.
- A signature only counts if the patient understands. Use an interpreter when needed.

CHECK YOURSELF

? **The patient is tired and says, 'Just sign for me.' May you?**

Answer: No. You can never sign for a patient. Use the option for patient cannot sign, write the reason in the note, and tell your supervisor.

? **When do you ask for the signature: at the start or the end of the visit?**

Answer: At the end, after care is finished.

? **A patient speaks only Spanish. The form is in English. What do you do?**

Answer: Do not ask the patient to sign something they cannot understand. Use an interpreter or a form in the patient's language, and tell your supervisor.

12

For Home Health Aides and CNAs

◆ Aides

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Use your task list correctly at every visit
- ✓ Notice and report changes in a patient
- ✓ Know what is, and is not, part of your job
- ✓ Understand aide supervision and why it helps you

Aides spend more time with patients than anyone else on the team. You are the eyes and ears of the care team. What you notice, and what you report, can protect a patient's health.

WORDS TO KNOW

Aide

A home health aide (HHA) or certified nursing assistant (CNA).

Care instructions

The written instructions a nurse, or another skilled professional, prepares for the aide. They list what to do for each patient.

Task

One thing the care instructions ask you to do.

Supervisory visit

A visit by a nurse or another skilled professional to check the aide's care.

Your job in Alora

1



Know your visits

Read your schedule. See Chapter 6.



2



Clock in and out

At the home, at the real time. See Chapter 8.



3



Do and mark tasks

Only tasks that are on your list.



4



Write and report

Note what you did and saw. Call if something changes.

Your task list

Each visit has a list of tasks. The tasks come from the care instructions written by a nurse or another skilled professional. Examples: help with bathing, dressing, walking, meals, and moving safely.

- Do the tasks on the list, in the way the patient wants, when that is safe.
- Mark a task **done** only when you did it.
- If a task was **not done**, write why. For example: "Patient refused bath. Offered again. Patient said no. Told supervisor at 9:15."
- Never do a task that is not on the list, unless your supervisor says yes.

Notice and report

You see the patient closely. You often notice small changes first. A small change can be a sign of a big problem.



Trouble breathing, chest pain, or very fast or slow pulse



A fall, or a new injury or bruise



New confusion, sleepiness, or a big change in behavior



A new sore, red skin, swelling, or a change in a wound



Eating or drinking much less, vomiting, or diarrhea



New pain, fever, or feeling much weaker than before

1

Stay with the patient and keep them safe

Do not leave the patient alone if they are not safe.

2

In an emergency, call 911

Chest pain, trouble breathing, a serious fall, or loss of consciousness are emergencies. Call 911 first. Then call the office.

3

For any other change, call the nurse or your supervisor now

Do not wait until the end of the visit, or the end of the day. Use the number in Appendix A.

4

Write what you saw and who you told

Write facts, the time, and the name of the person you told.

What is not your job

Every role has limits. The limits protect patients. If a family member asks you to do something outside your task list or your training, say kindly: "I am only allowed to do the tasks on my list. I will call my supervisor." Then make the call.

✓ DO

- ✓ Do the tasks on your list.
- ✓ Tell the nurse what you see.
- ✓ Ask when you are not sure.
- ✓ Treat the patient with respect and keep their dignity.

× DO NOT

- × Do not do tasks outside your list or your training.
- × Do not give medical advice or change what the nurse ordered.
- × Do not borrow money, or accept money or valuable gifts from a patient. Follow the Zenith Code of Conduct.
- × Do not leave before the visit ends, unless the office says you may.

Supervision of aides

FEDERAL RULE

A nurse or another skilled professional who is familiar with the patient must supervise aide services. If the patient also receives skilled care, this must happen at least **every 14 days**. If the patient receives only aide services, the nurse must visit the home at least **every 60 days**, to watch and assess each aide giving care. (42 CFR 484.80)

A supervisory visit is not a test. It helps the nurse see what you need, such as training, supplies, or clearer instructions. It also protects you, because the nurse sees that you follow the plan. The office tracks when supervisory visits are due. You do not need to ask for one. If a nurse comes while you are working, continue your care as usual.

Common situations

WHAT HAPPENS	WHAT YOU DO
The patient says, "I do not want a bath today."	Do not force. Ask kindly if later is better. If the answer is still no, write it down and tell your supervisor.
The patient asks you for a medicine.	Do not hand out or decide on medicines beyond what your training and the plan allow. Call your supervisor.
The patient falls during your visit.	Keep the patient safe and still. Call 911 if hurt. Call your supervisor. Write what you saw. Fill out an incident report as Zenith instructs.

WHAT HAPPENS	WHAT YOU DO
The patient's family asks for extra help, such as cleaning the whole house.	Say you can only do the tasks on your list. Offer to tell your supervisor about the request.
You think a patient is being hurt or neglected.	Report it right away. See Chapter 21.
You cannot finish all tasks in the time.	Do the most important and safest ones first. Write which tasks were not done and why. Tell your supervisor.

☆ REMEMBER

- Do the tasks on your list. Mark a task done only if you did it.
- If a task is not done, write why.
- Report changes right away. Call 911 for an emergency.
- Stay inside your job. When you are not sure, call.
- A nurse supervises aide care at least every 14 days (with skilled care) or every 60 days (aide services only).

CHECK YOURSELF

? The patient's ankles are more swollen than last week. What do you do?

Answer: Call the nurse or your supervisor now. Do not wait until the end of the day. Write what you saw, the time, and who you told.

? A family member asks you to do a task that is not on your list. What do you say?

Answer: "I am only allowed to do the tasks on my list. I will call my supervisor." Then call your supervisor.

? How often must a nurse supervise aide care when the patient also receives skilled care?

Answer: At least every 14 days.

13

For Nurses, Therapists, and Social Workers

● Clinicians

About 10 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Follow the timeline from first visit to recertification
- ✓ Finish the assessment (OASIS) and plan of care on time
- ✓ Handle orders, missed visits, and changes in condition correctly
- ✓ Supervise aides and coordinate care

Clinicians carry the clinical record. Your assessments and plans of care decide the care a patient gets, and the payments Zenith receives. This chapter follows the path a clinician takes with one patient. Many deadlines are federal rules, so they are fixed.

WORDS TO KNOW

OASIS

A standard set of questions about the patient's health and abilities. It is part of the comprehensive assessment.

Comprehensive assessment

The full, careful check of the patient's needs. It includes OASIS for patients who need it.

Plan of care

The written plan for one patient. The doctor signs it. All visits follow it.

Certification period

Up to 60 days covered by one plan of care.

Recertification

The new assessment at the end of a 60-day period, to decide if care continues.

The timeline

One 60-day certification period

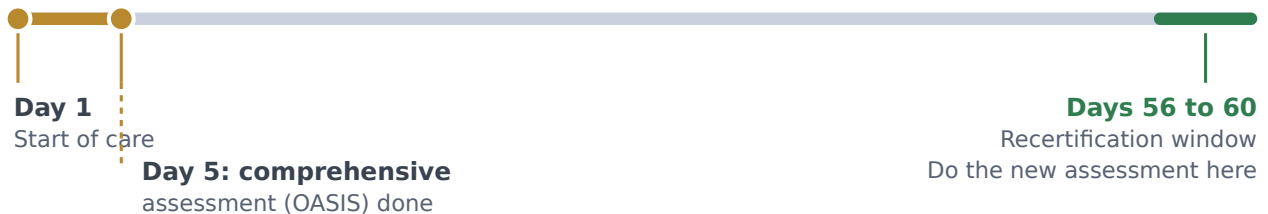
Days 1 to 60: the plan of care covers this whole time

Two 30-day billing periods

Period 1: days 1 to 30

Period 2: days 31 to 60

Key dates



Day 61 starts a new 60-day period

The 60-day period, with the two 30-day billing periods inside it. Day 1 is the start of care day. This picture follows the federal rules for Medicare. Other payers can have other rules. The office tells you the exact dates for each patient.

Step 1. The initial assessment visit (RN)

A registered nurse does the first visit. The nurse checks the patient's immediate needs, explains the patient's rights, and, for Medicare, checks the patient's eligibility for the home health benefit, including homebound status.

FEDERAL RULE

The initial assessment visit must happen within **48 hours** of the referral, or within 48 hours of the patient's return home, or on the start of care date the doctor ordered. (42 CFR 484.55)

☐ You have the doctor's order, and it matches the referral.

- ☐ You explained the patient's rights and gave the written notice before care.
- ☐ You checked the patient's needs, safety, and medicines.
- ☐ You set the start of care date in Alora.

Step 2. The comprehensive assessment and OASIS

After the first visit, the clinician completes the comprehensive assessment. For most adult patients who receive skilled care through Medicare or Medicaid, this includes the OASIS. Patients under 18, patients receiving maternity services, and patients receiving only personal care or housekeeping are not part of OASIS. The office will tell you when OASIS is required.

FEDERAL RULE

The comprehensive assessment must be completed no later than **5 calendar days** after the start of care. (42 CFR 484.55)

ZENITH STANDARD

Zenith target: finish it within **3 calendar days** of the start of care date. That leaves time to fix problems before the legal limit.

1

Answer every OASIS item from what you see and learn

Answer from what you observe, measure, and ask. Do not guess, and do not copy from an old assessment.

2

Run the OASIS check in Alora

Alora can check (validate) OASIS answers and has a tool, sometimes called an analyzer or scrubber, that looks for errors before you submit. Run it. Fix every error and question.

3

Submit it for review

The clinical reviewer checks it. The office then sends the OASIS data to the government.

FEDERAL RULE

The agency must send each completed OASIS to the federal system within **30 days** of completing the assessment. (42 CFR 484.45) The office does this. Your job is to complete it accurately and on time.

When is an OASIS required?

TIME POINT	WHEN IT HAPPENS	DUE
Start of care (SOC)	The first visit that begins care	No later than 5 calendar days after the start of care
Resumption of care (ROC)	The patient comes home after a hospital or facility stay of 24 hours or more	Within 48 hours of the patient's return home
Recertification	Near the end of each 60-day period	In the last 5 days of the period (days 56 to 60)
Other follow-up	A major change in the patient's health or needs, as the agency decides	Follow the OASIS guidance. Your supervisor will tell you the date.
Transfer to a facility	The patient goes into a hospital or facility	Follow the OASIS guidance. Your supervisor will tell you the date.
Death at home	The patient dies while receiving care	Follow the OASIS guidance. Your supervisor will tell you the date.
Discharge	Care with Zenith ends	Follow the OASIS guidance. Your supervisor will tell you the date.

For the exact rules for each time point, use the current CMS OASIS Guidance Manual. The office keeps the current version.

Step 3. The plan of care

The plan of care says who will give care, what they will do, how often, and the goals. Alora can build the standard CMS-485 from the OASIS answers, and can also offer a "living" plan of care that is easier to update. You review it, correct it, and send it to the doctor.

- Check that the diagnoses, orders, medicines, goals, and visit frequency are correct.
- Make sure the doctor's information is correct. Alora can fill in doctor details from the national NPI register.
- Send it to the doctor. Alora has electronic fax built in.
- Give care as the plan says. A change in frequency or services needs an order.

FEDERAL RULE

The doctor and the agency must review and revise the plan of care as often as the patient's condition requires, and at least **once every 60 days**, starting from the start of care date. (42 CFR 484.60)



IMPORTANT

The final claim for Medicare cannot be billed until the doctor has signed and dated the certification and plan of care. So unsigned plans of care stop payment. See Chapter 18.

Orders

- **Written orders:** make sure they are in the record and match what you do.
- **Verbal orders:** if a doctor gives an order by phone, write it in Alora right away. Read it back to the doctor to check it. The doctor must later sign and date it. Accept verbal orders only if your role and Zenith policy allow it.
- **Face-to-face encounter:** for Medicare, the doctor, or an allowed practitioner, must have seen the patient. It must be no more than 90 days before, or up to 30 days after, the start of care, and related to why the patient needs home health. The office collects this. Tell the office if you learn that one is missing.

Visits, missed visits, and changes

- **Visit as ordered.** The plan of care sets how often. If you cannot visit, call the office right away (Chapter 6). Write the reason for every missed visit. Tell your supervisor, so the doctor can be told as the plan or policy requires.

- **Report changes.** If the patient's condition or needs change in a way that suggests the plan should change, the agency must alert the doctor promptly. Call your supervisor and the doctor.
- **Therapists reassess.** At least once every 30 days, for each therapy service given, a qualified therapist (not an assistant) must do the ordered service and reassess the patient's function. The therapist compares the result to earlier measures. The therapist then documents the result and the effect of the therapy.

Step 4. Recertification

Near the end of each 60-day period, the clinician does a new comprehensive assessment to decide whether care should continue.

FEDERAL RULE

The recertification assessment must be done in the last **5 days** of the 60-day period, which are days 56 to 60. (*42 CFR 484.55*)

ZENITH STANDARD

Zenith target: do it on **days 56 to 58**. This leaves time to fix problems before day 60.



ALORA PLUS HAS A CALCULATOR

If you use Alora Plus, it includes a **60 Day Episode Calculator** that works without internet. You enter the start of care date, and it shows the dates of the 60-day periods. Ask the office if your app has it.

Supervising aides

A nurse or another skilled professional who knows the patient must supervise aide services: at least **every 14 days** if the patient also receives skilled care, and at least **every 60 days** on site if the patient receives only aide services. Write the supervisory visit in Alora, and check that the aide's task list still matches the patient's needs. See Chapter 12.

Discharge

- Complete the discharge assessment and the discharge summary.

- Give the patient clear instructions, in a language they understand.
- Tell the office the discharge date and the reason, so the schedule, the orders, and the billing can be closed.

Notes for therapists and social workers

Therapists and social workers also write visit notes and assessments in Alora. The same rules apply. Write the same day. Write facts. Use the plan of care goals. For therapists, include measurements that show progress. For social workers, include the needs you found, the referrals you made, and the follow-up plan.

☆ REMEMBER

- First visit within 48 hours of referral (or the ordered date). Full assessment within 5 calendar days of start of care.
- Run the OASIS check in Alora and fix every error before you submit.
- The doctor reviews the plan of care at least every 60 days. The final Medicare claim needs the signed plan.
- Recertify in the last 5 days of each 60-day period (days 56 to 60).
- Supervise aides every 14 days (with skilled care) or every 60 days (aide services only).
- Therapists reassess at least every 30 days.

CHECK YOURSELF

? The referral came on Monday at noon. By when must the initial assessment visit be done?

Answer: Within 48 hours of the referral, so by Wednesday at noon, unless the doctor ordered a different start of care date.

? The start of care is January 1. What is the latest day to finish the comprehensive assessment?

Answer: No later than 5 calendar days after the start of care, which is January 6. The Zenith target is earlier.

? Which days are the recertification window in a 60-day period?

Answer: Days 56 to 60 (the last 5 days).

? A doctor gives you an order by phone. What do you do?

Answer: Write it in Alora right away and read it back to the doctor. The doctor must later sign and date it. Only accept verbal orders if your role and Zenith policy allow it.

PART C

In the Office

These six chapters are for the office team: intake, scheduling, monitoring, clinical review, billing, and administration. Field staff may read them to understand who is waiting for their work, and why.

14

Intake and New Patients

■ Office team

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Enter a new patient correctly from a referral
- ✓ Start the 48-hour clock with the exact time of the referral
- ✓ Check insurance, doctors, and orders before the first visit
- ✓ Prevent duplicate and wrong records

Intake is the front door of Zenith. Every patient record begins here. A small mistake at intake, such as a wrong digit in an insurance number, can travel through the whole record and stop payment months later. Take your time and check each detail.

WORDS TO KNOW

Referral

A request for Zenith to care for a patient.

Intake

The office steps that bring a new patient into Zenith.

NPI

National Provider Identifier. A 10-digit number for a health care provider, such as a doctor.

Eligibility check

A check with the payer that the patient's insurance is active and covers home health.

Prior authorization

Permission from a payer before care begins.

The intake steps

1 Receive the referral and record the exact time

Write the date and time the referral was received, and who sent it. This starts an important clock. The first nurse visit must happen within 48 hours of the referral, or on the date the doctor ordered.

2 Search before you create

Search Alora for the patient by last name and birth date. The patient may already have a record from an earlier stay. Never make a second record for the same person. Duplicate records split a patient's history in two.

3 Create the patient record

Enter the details exactly as they appear on the patient's insurance card and identification: name, birth date, address, phone, emergency contact, and preferred language. Ask about safety notes for the home, such as pets or a gate code.

4 Add the doctor

Alora can bring in a doctor's details from the national NPI register when you add a doctor. Check the name, NPI, phone, and fax number against the referral. A wrong fax number sends private information to the wrong place.

5 Check insurance

Confirm that the insurance is active, and find out if the payer needs prior authorization. Record the payer, the member number, and the date you checked. If something does not match, stop and ask.

6 Collect orders and documents

Get the doctor's order and the referral papers. For Medicare, also get proof of the face-to-face encounter. Save every document in the patient's record.

7 Decide if Zenith can take the patient

The Director of Nursing or the Administrator decides if Zenith can safely and legally care for the patient: services needed, service area, staffing, and payer. Do not promise

care before this decision.

8

Plan the first visit

Work with the scheduler to book the initial assessment visit with an RN, within the time limit. Tell the patient or family the date and time.

FEDERAL RULE

The initial assessment visit must happen within **48 hours** of the referral, or within 48 hours of the patient's return home, or on the start of care date the doctor ordered. (42 CFR 484.55)

Get the details right

DETAIL	WHY IT MATTERS	HOW TO CHECK
Name and birth date	Payers match claims to the person. A wrong letter can cause a denial. It can also put a note in the wrong chart.	Copy from the insurance card and an ID. Read it aloud to the patient or family.
Insurance number	One wrong digit makes the claim fail.	Check twice. Check eligibility in the payer's system.
Address and phone	The team member must find the home. The clock in location can be checked against the address.	Confirm the address with the patient. Use the exact street and unit number.
Preferred language	Patients have the right to be informed in a language they understand.	Ask. Record it. Plan an interpreter if needed.
Doctor and NPI	Orders and plans of care must go to the right doctor.	Use the NPI lookup. Check the fax number.
Emergency contact	The team needs someone to call in an emergency.	Ask who may be told about the patient's care, and record it.

Privacy at intake

Referral papers are full of PHI. Keep them private.

- Before you fax, check the fax number digit by digit. A fax to the wrong number is a privacy incident. Report it (Chapter 3).
- Do not leave papers on a desk or in a shared printer.
- Do not discuss referrals where visitors or other people can hear.

New patient checklist

- ☐ Referral time and source are recorded.
- ☐ The patient was searched for, and there is no duplicate.
- ☐ Name, birth date, address, phone, and language are checked.
- ☐ Doctor is added, with NPI and fax number checked.
- ☐ Insurance is checked. Authorization is requested if needed.
- ☐ Doctor's order and referral papers are in the record.
- ☐ Face-to-face proof is requested (Medicare).
- ☐ The Director of Nursing or Administrator approved the admission.
- ☐ The initial assessment visit is booked within the time limit.
- ☐ The patient or family knows the date and time.

☆ REMEMBER

- Record the exact date and time of the referral. The first RN visit is due within 48 hours.
- Search first. Do not create duplicate records.
- Copy names and numbers exactly from cards and IDs. Check twice.
- Check eligibility and authorization before care starts.
- Check fax numbers digit by digit.

CHECK YOURSELF

? A referral arrives Friday at 4:30 PM. By when must the nurse visit?

Answer: Within 48 hours, which is Sunday at 4:30 PM, unless the doctor ordered a different start of care date. The scheduler must plan for the weekend.

? You cannot find the patient in a search. What do you do before creating a new record?

Answer: Search again with another spelling, the birth date, and the phone number. Ask a colleague. Create a new record only when you are sure the patient is not in Alora.

? Why is a wrong fax number a problem?

Answer: It sends private patient information to a person who should not have it. That is a privacy incident and must be reported.

15

Scheduling and Staffing

■ Office team

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Build a schedule that follows the plan of care
- ✓ Choose the right team member for each visit
- ✓ Never schedule someone whose credential has expired
- ✓ Handle changes, cancellations, and open visits

The schedule is the promise Zenith makes to each patient. When the schedule is right, patients get care on the days the doctor ordered, team members know where to go, and the records match. This chapter shows how to build and keep a good schedule.

WORDS TO KNOW

Visit frequency

How often a service visits, written in the plan of care. Example: 2 visits a week for 4 weeks.

Credential

A license, certification, or training that a team member must have for a job.

Open visit

A planned visit that has no team member yet.

Conflict

Two visits for one team member at the same time, or a visit that breaks a rule.

What Alora can do for scheduling

- Show the schedule from three points of view: one **patient**, one **team member**, or the whole **agency**.
- Show it by **day, week, month, or certification period**.
- Enter many visits at once in a **batch entry** table of rows and columns.
- Warn you about **conflicts** and compliance problems, such as authorizations and visit frequency.
- Send **automatic notices** about open visits to team members.

Build the schedule step by step

- 1 Start from the plan of care**

Read the orders. They say which service goes, how often, and for how long. Example: skilled nursing 2 times a week for 4 weeks. Schedule exactly this.
- 2 Pick the days and times with the patient**

Ask the patient or family what times work. A fixed day each week helps patients plan and helps your team.
- 3 Create the visits**

Use the batch entry if you have many visits for one patient. Check that the dates fall inside the certification period and the authorization.
- 4 Assign a team member**

Choose the right person. See below. If no one is free, post the visit as an open visit.
- 5 Check for warnings**

Read every warning Alora shows. Fix conflicts before you save. Do not ignore a warning because it appeared before.
- 6 Tell the team member and the patient**

Make sure the team member sees the visit. Tell the patient the day and time.

Choose the right team member

1



The right skill

An RN for skilled nursing. An aide for personal care. A therapist for therapy.



2



A valid credential

License and certification are current. Nothing has expired.



3



Close enough

The travel is realistic. There is time between visits.



4



A good fit

Same team member when possible. The patient's language and needs.



NEVER SCHEDULE AN EXPIRED CREDENTIAL

Before you assign a team member, check that their license, certification, and required health items are current. Zenith keeps this information in the **Zenith HR Command Center**, and Alora can also track credentials and expiration dates. If anything has expired, do not assign the person to a visit. Tell the Administrator.

Keep the schedule correct

CHANGE	WHAT TO DO IN ALORA	ALSO DO
The patient cancels a visit.	Record the reason. Mark the visit as cancelled. Do not delete it.	Tell the clinical supervisor the same day. The doctor may need to be told, and a make-up visit may be needed.
A team member calls out.	Reassign the visit, or post it as an open visit.	Call the team member who takes it. Tell the patient if the time changes.
The patient asks to move a visit.	Move the visit. Check the plan of care frequency.	Tell the team member. Record the reason.
A new order changes the	Update the visits to match the new order.	Check that the order is in the record first.

CHANGE	WHAT TO DO IN ALORA	ALSO DO
frequency.		
A visit was missed.	Record that it was missed, and why. Do not delete it.	Tell the clinical supervisor the same day.



DO NOT DELETE VISITS

Mark a visit as cancelled or missed and record the reason. A deleted visit leaves a hole in the record, and payers and auditors look for holes.

Open visits

- 1 Post the open visit**
 Make sure the day, time, address, and service are correct. Alora can notify team members who may take it.
- 2 Watch who responds**
 Check that the team member has the right credential and is free.
- 3 Confirm the visit**
 Check that the visit now shows the team member's name. Tell the patient if the team member is new.
- 4 If nobody takes it, escalate early**
 Do not wait until the last hour. Tell the clinical supervisor. A missed visit is a problem for the patient and for the record.

The schedule check

Do this check at the end of every day for tomorrow, and at the end of every week for next week. It takes a few minutes and prevents most missed visits.

- ☐ Every visit has a team member assigned.
- ☐ No team member has two visits at the same time.

- ☐ Travel time between visits is realistic.
- ☐ Every assigned team member has current credentials.
- ☐ Visits match the ordered frequency for each patient.
- ☐ Visits fall inside the certification period and the authorization.
- ☐ Every open visit has a plan.
- ☐ Patients whose visits changed have been told.

☆ **REMEMBER**

- Schedule exactly what the plan of care orders.
- Check credentials before you assign. Never schedule an expired credential.
- Read every warning Alora shows.
- Cancel or mark missed, with a reason. Never delete a visit.
- Do the schedule check every evening and every week.

CHECK YOURSELF

? A nurse's license expired yesterday. May you assign that nurse to tomorrow's visits?

Answer: No. Do not assign a team member with an expired credential. Tell the Administrator, and find another team member for the visits.

? A patient cancels a visit. Do you delete it from the schedule?

Answer: No. Mark it as cancelled and record the reason. Tell the clinical supervisor the same day.

? Alora shows a warning that a visit breaks the ordered frequency. What do you do?

Answer: Read the warning and fix the cause. Do not save the visit and ignore the warning. Check the plan of care and the orders.

16

Live Monitor and EVV Exceptions

■ Office team

About 8 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Read the Live Monitor and act on each color
- ✓ Respond to a late visit or a no-show step by step
- ✓ Review and fix EVV exceptions honestly
- ✓ Keep a clean record of every change you make

The Live Monitor is the office's window into the field. It shows visits as they happen. When you watch it well, late visits and no-shows are found in minutes, not hours. This chapter shows how to watch, how to respond, and how to fix problems in the record with honesty.

WORDS TO KNOW

Live Monitor

An Alora screen that shows visits as they happen, with color warnings.

No-show

A visit where the team member did not arrive or clock in.

Exception

A visit record that does not match the plan and needs office review.

Audit trail

The history of who changed a record, what they changed, and when.

The Live Monitor

Alora's Live Monitor shows every visit's status in real time. Colors warn about delays and no-shows. Alora can also send text or push notices about delays and no-shows to chosen office team members and team members' phones, if Zenith turned this on.

Live Monitor (simplified example)			
PLANNED	TEAM MEMBER	PATIENT	STATUS
8:00 AM	Aide A	J. D.	✓ In progress, on time
9:00 AM	Nurse B	M. R.	⌚ Late, not clocked in
9:30 AM	Aide C	A. L.	⚠ No-show
7:00 AM	Therapist D	S. T.	✓ Complete

Sample data. Screens, words, and colors can be different in your Alora.



DO NOT WAIT FOR AN ALERT

Alerts are a help, not a guarantee. Phones can be off, and notices can be delayed. Check the Live Monitor yourself at regular times, such as at the start of every hour, and more often in the busy morning hours. Do not wait for an alert to tell you that a visit is late.

When a visit is late

A late visit is a patient who is waiting. Act quickly and calmly. Always ask first if the team member is safe.

1



Yellow warning

Visit starts late. Wait the set time.



2



Call the team member

Ask if they are safe and where they are.



3



Red warning

No contact. Start the backup plan.



4



Write it down

Record every call: time, person, result.

ZENITH STANDARD

Call the team member when a visit has not started **15 minutes** after the planned time. Start the backup plan when there is no contact **30 minutes** after the planned time.

The call to the team member

Say: "Hello, this is [your name] from the Zenith office. Your visit with [patient] was planned for [time]. Are you safe? Where are you now? When can you arrive?"

Ask about safety first. Then ask about the visit. Keep a kind voice. Late arrivals often have a simple reason, such as traffic or a problem in the last home.

The backup plan

1

Try to reach the team member again

Call, then text if Zenith allows, then call the emergency contact in the team member's record if you are worried about their safety.

2

Reach the patient or family

Tell them the visit is late and give a new time, or tell them that another team member is coming.

3

Find another team member

Post the visit as an open visit, or call the on-call team member. Tell your supervisor.

4

If a patient may be in danger, ask for a welfare check

Sometimes the team member is at the door and the patient does not answer.

Sometimes you cannot reach a patient who is at risk. In both cases, call 911 and ask for

a welfare check.

5

Record what happened

Write each call and action with the time. If a visit was missed, record it as missed with the reason, and tell the clinical supervisor so the doctor can be told as needed.

EVV exceptions

Alora flags visits that do not match the plan. For example, the team member forgot to clock out. Alora holds these visits for manual review and approval before it sends the visit data onward. This review is your job. The exact list of things that get flagged depends on how Zenith's Alora is set up. These are common examples.

COMMON EXCEPTION	WHAT IT MAY MEAN	WHAT TO DO
No clock out	The team member forgot, or had a problem.	Ask the team member when care really ended. Check the note.
No clock in	The team member forgot, or the clock in did not sync.	Ask. Check if the phone is offline and not synced.
Clock in far from the patient's address	The GPS was wrong, or the address is wrong, or the team member was not there.	Ask the team member. Check the patient's address. Never approve a visit without an answer you believe.
Visit much shorter or longer than planned	The patient ended it early, or care took longer, or the times are wrong.	Ask. Read the note. Record the real reason.
Telephone method was used	The app could not be used.	Check that the team member followed the steps, and that the visit details are right.

Review an exception step by step

1

Open the exception and read what does not match

2

Ask the team member what really happened

Ask open questions. Do not suggest an answer.

3

Check the note and other records

Does the note agree? Do times in the note match?

4

Decide

Accept the record as it is, or correct it to what really happened. If you are not sure, ask the supervisor.

5

Write a clear reason, then approve

The reason should answer: what was wrong, how do we know, and who told us.

✗ POOR REASON

"Fixed time."

A reviewer cannot tell what was wrong or why the change is true.

✓ GOOD REASON

"Team member forgot to clock out. Team member called at 11:20 and said the visit ended at 10:45. The note is signed at 10:47. Clock out entered as 10:45, as reported by the team member."

It says what happened, how we know, and who reported it.

✓ DO

- ✓ Record only what really happened.
- ✓ Ask the team member. Write down the answer and the time you asked.
- ✓ Review exceptions within **1 business day**.
- ✓ Send serious concerns to the Compliance Officer.

× DO NOT

- × Do not change times to match the schedule, or to make a visit look better.
- × Do not approve an exception you do not believe.
- × Do not let team members tell you what to write to avoid a problem.
- × Do not edit without a reason.

FEDERAL RULE

Federal and Florida law treat knowingly false records and false billing as fraud. An edit that makes a visit look different from what really happened can put a person and Zenith at serious risk. If you see a pattern, such as the same team member with many exceptions, tell the Compliance Officer.

After approval

Approved visits are sent onward, as Zenith has set it up. For Medicaid visits, this means the state EVV system. Some visits can be rejected there, for example because of a mismatch in patient or provider numbers. Check regularly for rejected visits and fix them, using the same honest process.

☆ REMEMBER

- Check the Live Monitor yourself. Do not wait for an alert.
- Late visit: call, ask if the team member is safe, then start the backup plan on time.
- If a patient may be in danger, call 911 for a welfare check.
- Review exceptions by asking what really happened. Record only the truth.
- Every edit needs a clear reason. Never change times to fit the schedule.

CHECK YOURSELF

? A visit turns yellow. What is your first action?

Answer: Call the team member. Ask first if they are safe, then ask where they are and when they can arrive.

? A team member says, 'Just put my clock out at 11:00, I do not remember exactly.' What do you do?

Answer: Do not enter a time you do not believe. Check the note and other records, ask again, and record the real time as best as it can be known. If it cannot be known, record that and the reason, and tell your supervisor.

? Why is 'Fixed time' a poor reason for an edit?

Answer: It does not say what was wrong, how we know, or who said so. A reviewer could think the change was made to hide something.

17

Clinical Review and Quality Checks

■ Office team

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Know what a clinical reviewer checks, and when
- ✓ Track orders, signatures, and due dates so nothing is missed
- ✓ Send records back for correction in a clear and kind way
- ✓ Recognize warning signs of false or copied records

Quality review catches problems while they are still small. A missing signature found today takes five minutes to fix. The same signature, found during an audit a year later, can cost money and trust. This chapter shows what to check, when to check it, and how to ask for a fix.

WORDS TO KNOW

QA

Quality Assurance. Checking records for mistakes and missing items.

Audit

A careful review of records by Zenith, a payer, or the government.

Chart

The full record of one patient.

Scrubber

A tool in Alora that looks for errors before an OASIS or a claim is sent.

What the reviewer checks

Think of the review as a set of questions. Each question has a right answer and a deadline.

AREA	THE QUESTION	THE DEADLINE
First visit	Did an RN do the initial assessment visit? Was the patient's rights notice given?	Within 48 hours of referral, or the ordered date
OASIS	Is the comprehensive assessment complete? Did it pass the Alora OASIS check (scrubber)? Was it sent to the government system?	Complete within 5 calendar days of start of care. Send within 30 days of completion.
Plan of care	Does it match the OASIS and the orders? Has the doctor signed and dated it?	Reviewed at least every 60 days
Orders	Is every order in the record? Is every verbal order signed and dated by the doctor?	Follow up when orders are unsigned (7 calendar days)
Face-to-face	Is there a face-to-face encounter for Medicare, related to the reason for home health?	From 90 days before to 30 days after the start of care
Visit notes	Is there a note for every visit? Is it signed, complete, and written on time? Do vital signs and tasks look complete? Are missed tasks explained?	Same day (the end of the same day)
Visit match	Do the clock in and out times match the note and the schedule?	Daily
Frequency	Did visits happen as often as ordered? Are missed visits explained, and was the doctor told?	Weekly

AREA	THE QUESTION	THE DEADLINE
Aide supervision	Is a nurse supervisory visit done and recorded on time?	Every 14 days (skilled care) or every 60 days (aide services only)
Therapy	Did a qualified therapist reassess the patient?	At least every 30 days
Recertification	Is the recertification assessment done inside the window?	Days 56 to 60 (Zenith target: days 56 to 58)
Incidents	Was every incident reported and followed up?	As soon as it happens

A daily review routine

- 1 Start with the newest admissions**
 New patients carry the strictest deadlines. Check the 48-hour visit and the 5-day assessment first.
- 2 Work the oldest items next**
 Open items get harder to fix with time. Work the oldest first.
- 3 Review the notes from yesterday**
 Check that each visit has a signed note, and that the times match the EVV record.
- 4 Send back what needs a fix**
 Use the steps below. Keep a list of what you sent back and when.
- 5 Follow up on what was sent earlier**
 Check unsigned orders, plans of care, and returned notes. Write who you contacted and when.

How to ask for a correction

The goal is an accurate record and a team member who learns. Be specific, kind, and focused on the record, not the person.

✗ **LESS HELPFUL**

"Note is wrong. Fix it."

The team member does not know what is wrong.

✓ **MORE HELPFUL**

"In your 9/14 note for J. D., the blood pressure is missing. Please add it if you measured it. If you did not, write that it was not taken and why."

It names the note, the missing item, and the honest options.



NEVER ASK A TEAM MEMBER TO WRITE SOMETHING THEY DO NOT KNOW

If a number or fact is missing and the team member does not remember it, the answer is "not recorded", with the reason. It is never a guess, and it is never a number copied from another visit.

Track unsigned orders and plans of care

The doctor must sign the plan of care and any verbal orders. The final Medicare claim cannot be billed without a signed plan of care and certification. Keep a list of everything you sent and everything you are waiting for.

- 1 Send and record**
Send by e-fax or through the physician portal if Zenith uses it. Record the date and time.
- 2 Follow up on schedule**
If nothing comes back after 7 calendar days, call the doctor's office and send again.
- 3 Escalate**
If it is still missing, tell the Director of Nursing. The Director of Nursing can call the doctor directly.

Warning signs

Sometimes a record is not just incomplete. It is not true. Reviewers are the first line of defense. Watch for these signs, and tell the Compliance Officer if you see them. Do not accuse anyone. Report what you see.



Notes for different visits that are word for word the same



Note times or clock times that do not match the schedule or each other



Visits recorded when the patient was in the hospital



Signatures that look identical on many days



Notes written long after the visit, made to look on time



Many EVV edits for the same team member

☆ REMEMBER

- Review in order: new admissions first, then the oldest open items.
- Know the deadlines: 48 hours, 5 days, 30 days, 14 and 60 days, 30-day therapy reassessments.
- Ask for corrections with clear, specific, kind words.
- Never ask anyone to write a guess. "Not recorded" is an honest answer.
- Track unsigned orders until they come back. The final claim depends on them.
- Report warning signs to the Compliance Officer.

CHECK YOURSELF



A visit note has no blood pressure, and the team member does not remember it. What should the record say?

Answer: That the blood pressure was not recorded, with the reason if known. Never a guess, and never a number from another day.



Two visit notes on different days are word for word the same. What do you do?

Answer: Do not accuse. Tell your supervisor or the Compliance Officer what you saw. Copied notes can be false records.

? What is the deadline to send OASIS data to the government system?

Answer: Within 30 days of completing the assessment.

18

Billing and Getting Paid

■ Office team

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Follow a claim from the first visit to the payment
- ✓ Send the Notice of Admission on time
- ✓ Check that a claim is ready before you send it
- ✓ Work denials and follow up on money that is late

Billing turns care into payment. Zenith depends on payers to pay for the care it gives. Billing depends on everyone else: the visit must have happened, the record must prove it, and the doctor's signature must be in place. This chapter shows the path.

WORDS TO KNOW

Payer

The company or program that pays for care: Medicare, Medicaid, an insurance plan, or the patient.

Claim

A bill sent to a payer.

PDGM

The way Medicare pays home health. It pays per 30-day period, and groups patients by their needs.

NOA

Notice of Admission. A one-time message to Medicare that a home health admission has started.

Remittance

The payer's report that says what it paid and what it did not.

Denial

When a payer refuses to pay a claim.

The road from visit to payment

1

Send the Notice of Admission (NOA)

For Medicare, send the NOA soon after the start of care.

FEDERAL RULE

The NOA is due within **5 calendar days** of the start of care. If it is late, Medicare reduces the payment. The cut is one thirtieth of a 30-day payment for each day that the NOA is late. Those days cannot be billed. A late NOA costs real money.

2

Care is given and documented

Every visit has a clock in and out, a signed note, and the patient's signature when needed. This is what your claim rests on.

3

The assessment is complete

The OASIS is complete, checked, and sent. Medicare sets the payment group from this information.

4

The plan of care and certification are signed

The doctor has signed and dated them, and the face-to-face encounter is on file.

5

Check the claim is ready

Use the checklist below. Use any claim checking tools that Zenith's Alora offers.

6

Send the claim

Alora can prepare paperless claims for Medicare, Medicaid, other insurance, and private pay.

7

Post the payment

Read the remittance. Record what was paid. Mark what was not paid.

8

Work denials and follow up

Find out why a claim was denied, fix it, and send it again before the payer's deadline.

How Medicare pays, in short

Under PDGM, Medicare pays home health for each **30-day period**. A normal 60-day certification period therefore holds two 30-day billing periods. You saw this in the timeline in Chapter 13.

1



60-day period

One plan of care covers up to 60 days.



2



Two 30-day claims

Medicare pays per 30 days.



3



New period

After day 60, a recertification starts a new period.

Other payers have their own rules. Medicaid, managed care plans, and private insurance each have their own forms, approvals, and deadlines. Keep a list of each payer's rules.

Is the claim ready?

Do not send a claim until every answer is yes. A claim sent too early will be denied or, worse, paid incorrectly.

☐

The NOA was sent and accepted in time (Medicare).

☐

Every visit on the claim has a clock in and out, and the times match the note.

- ☐ Every visit has a signed, complete note.
- ☐ Visits match the plan of care frequency, or the missed visits are explained.
- ☐ The OASIS is complete and sent.
- ☐ The plan of care and certification are signed and dated by the doctor.
- ☐ The face-to-face encounter is on file (Medicare).
- ☐ Insurance is active. Authorization is in place if needed.
- ☐ Name, birth date, and insurance number match the payer's records.
- ☐ Diagnosis codes and dates are correct.
- ☐ For Medicaid, the visits on the claim match the EVV records.

FEDERAL RULE

Bill only for care that was given, and that the record proves. Billing for a visit that did not happen, or for a service that was not given, is fraud.

Why claims are denied

COMMON CAUSE	HOW TO PREVENT IT
Wrong name, birth date, or insurance number	Check at intake. Check eligibility before each new period.
Late or missing NOA	Send it within 5 calendar days of the start of care.
Plan of care or certification not signed	Track unsigned orders every week (Chapter 17).
Face-to-face missing or not related	Ask for it at intake. Check that it matches the reason for care.
Missing authorization	Check at intake and when orders change.
Visit records do not match the claim (Medicaid EVV)	Review EVV exceptions quickly. Approve only true records.
Duplicate claim	Check before you send again.

When a claim is denied

- 1 Read the reason**
The remittance gives a reason. Write it down.
- 2 Find the cause in the record**
Look for the missing signature, the wrong number, or the missing visit.
- 3 Fix the cause honestly**
Correct a real mistake. If a required item does not exist, for example a visit that did not happen, do not create or change records to make the claim work. Tell your supervisor.
- 4 Send again or appeal before the deadline**
Each payer has a deadline. The office keeps a list.
- 5 Learn from it**
Tell the person who can prevent it next time: intake, a clinician, or the scheduler. Repeated denials from one cause show where training is needed.

Follow up on money

- Check the list of unpaid claims every week. Work the oldest first.
- Call the payer about claims with no answer after the usual waiting time.
- Record each call: date, person, and result.
- Tell the Administrator about payers that are very slow, or about patterns you notice.

☆ REMEMBER

- Send the NOA within 5 calendar days of the start of care. Late NOAs lose money.
- Medicare pays per 30-day period. A 60-day period has two.
- A claim is ready only when the visits, notes, OASIS, signed plan of care, and face-to-face are all in place.
- Bill only for care that was given and is proven by the record.
- Fix the real cause of a denial. Never change a record just to get paid.

CHECK YOURSELF

? The start of care was on the 1st. By when must the NOA be sent?

Answer: Within 5 calendar days, so by the 6th.

? The doctor has not signed the plan of care. Can you send the final Medicare claim?

Answer: No. The signed and dated plan of care and certification must be in place first. Follow up with the doctor's office.

? A claim for a visit was denied, because the record shows no clock in. The visit really happened. What do you do?

Answer: Find out the real facts from the team member, and let the EVV review correct the record honestly, with a clear reason. Then send the claim again. Never add a clock in that did not happen.

19

Daily, Weekly, and Monthly Routines

■ Office team

About 4 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Use a simple routine for each office role
- ✓ Do the end-of-day check every day
- ✓ Run the weekly and monthly checks that catch slow problems
- ✓ Know who to tell when a check finds something

Good offices do small things on a steady rhythm. These routines take little time, and they prevent the big problems. Use them as a starting point. The Administrator may adjust them for Zenith.



GOOD PRACTICE

The routines in this chapter are recommended practice. Zenith may change them. Your supervisor can tell you which ones are required.

Every day



Scheduler

- ☐ Morning: read overnight changes and call-outs.
- ☐ Watch the Live Monitor through the day.
- ☐ Fill open visits for tomorrow.
- ☐ Evening: run the schedule check (Chapter 15).



Intake

- ☐ Log each new referral with the exact time.
- ☐ Check the 48-hour visit for each new patient.
- ☐ Follow up on missing orders and authorizations.
- ☐ Send confirmations to new patients.



Clinical reviewer

- ☐ Review yesterday's visit notes.
- ☐ Check new admissions: 48 hours and 5 days.
- ☐ Work the EVV exception list.
- ☐ Follow up on returned corrections.



Billing

- ☐ Send any NOAs that are due.
- ☐ Post payments and remittances.
- ☐ Work new denials.
- ☐ Check claims that are ready to send.



Director of Nursing and Administrator

- ☐ Review incidents and urgent reports.
- ☐ Answer clinical questions from the field.
- ☐ Approve new admissions.

- ☐ Look at the day's problems and the plan for tomorrow.



Everyone

- ☐ Lock your screen when you leave your seat.
- ☐ Log out at the end of the day.
- ☐ Put papers with patient information away.
- ☐ Report privacy or safety problems at once.

Every week

- ☐ **Scheduler:** check next week's schedule for gaps, conflicts, and expired credentials.
- ☐ **Clinical reviewer:** check visit frequency for each patient, missed visits, aide supervisory visits due, and therapy reassessments due.
- ☐ **Clinical reviewer:** list all patients who will reach days 56 to 60 in the next two weeks, so recertifications can be planned.
- ☐ **Clinical reviewer:** follow up on every unsigned plan of care and order.
- ☐ **Billing:** review the list of unpaid claims and the oldest denials.
- ☐ **Intake:** check authorizations that will end soon.
- ☐ **Administrator:** review the list of open EVV exceptions and late notes, and talk with team members who need help.

Every month

- ☐ Audit a small sample of charts from each discipline: check the deadlines in Chapter 17.
- ☐ Compare EVV records, visit notes, and claims for a sample of visits.
- ☐ Check that team members' licenses, certifications, and health items are current, and plan for those that will expire soon.
- ☐ Review user accounts. Close accounts for people who left. Check that roles match jobs.
- ☐ Review denials by cause, and plan training for the most common cause.
- ☐ Review incidents, privacy reports, and complaints for patterns.

☐ Tell the Administrator what you found, and what you fixed.

When a check finds something

YOU FIND	TELL
A missed visit, a late patient, or a patient at risk	The clinical supervisor, the same day
An expired credential	The Administrator, the same day. Do not schedule that person.
A privacy problem (wrong fax, lost device)	Your supervisor and the Compliance Officer, at once
Signs of a false or copied record	The Compliance Officer
A billing pattern that seems wrong	The Administrator
A problem with Alora itself	Your supervisor. They contact Alora support.

☆ REMEMBER

- Small daily checks prevent large problems.
- Each role has a short daily list. Do it in the same order every day.
- Weekly checks find frequency, recertification, and unsigned order problems.
- Monthly checks find slow problems: credentials, accounts, denials, and patterns.
- Tell the right person the same day.

CHECK YOURSELF

? Which weekly check helps you not miss a recertification?

Answer: List patients who will reach days 56 to 60 in the next two weeks, so the assessments can be planned inside the window.

? You find a nurse with an expired license on next week's schedule. Who do you tell?

Answer: The Administrator, the same day. Do not schedule the nurse until the credential is current.

PART D

When Things Go Wrong

Problems are normal. What matters is what you do next. These three chapters give you answers for the most common problems, for emergencies and abuse reporting, and for the rules of honest records.

20

Problem Solver

● Everyone

About 9 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Find the answer to a common problem in under a minute
- ✓ Know who to call, and in what order
- ✓ Keep the record honest while you fix a problem
- ✓ Keep working safely when Alora is not available

Something did not work. Do not worry. Find your problem below and follow the steps. Each answer tells you what to do, and what to avoid. You can also use the search box.

Who to call, in order

The phone numbers are in Appendix A. Put them in your phone before your first visit.

1



Emergency

Call 911 first. Then the office.



2



Your supervisor

For any problem with a patient or a visit.



3



The office

For schedule, clocking, and Alora problems.





On-call nurse

After hours and on weekends.

Logging in and my account

▼ I forgot my password.

Tap **Forgot password?** on the login screen and follow the steps. If there is no such link, or it does not work, call the office for a reset. Do not ask a co-worker to log in for you.

▼ My login says the password or username is wrong.

Check the capital letters and the spaces. Try once more. If it fails again, stop and call the office. More tries can lock the account.

▼ My account is locked.

Call the office. The office can unlock it. Keep the patient's care going: your supervisor can tell you what to do until you can log in.

▼ I cannot open a screen that I need for my job.

Your role may not include that screen. Ask your supervisor. Do not use another person's account. The Administrator can change your role if your job needs it.

▼ I can see a patient who is not assigned to me.

Close it. Do not read it. Tell your supervisor, so they can check your role. Open only what you need.

▼ I got a new phone.

Call the office **before** you change phones. First sync everything on the old phone, and check that the waiting list is empty. Then install Alora on the new phone and log in.

The app on my phone

▼ The app will not open, or it closes by itself.

1. Close the app fully and open it again.

2. Restart the phone.
 3. If it still fails, use the telephone method for clocking in and out, and call the office.
 4. Do not delete the app. Unsent work could be lost.
-

▼ I do not get notices.

Open your phone settings. Check that notifications are on for Alora, that the phone is not on silent or do-not-disturb, and that battery saver is off. Also check your schedule in the app yourself. Do not wait for notices.

▼ The app says the location is off.

Open the phone settings, find Alora, and set location to **While using the app**. If there is a "Precise" switch, turn it on. See Chapter 4.

▼ My phone is full.

Delete old photos and unused apps. Do not delete Alora data. Keep enough space free so offline work can be saved.

▼ My phone is lost or stolen.

Call your supervisor right away. See Chapter 3. Do not wait.

My schedule

▼ A visit is not on my schedule.

Sync first. If it is still missing, call the scheduler. Do not go to a visit that is not on your schedule.

▼ A visit changed after I left home.

Read the new visit card. If the new time does not work for you, call the office at once. Tell them what you can and cannot do.

▼ The address is wrong, or I cannot find the home.

Call the office, and call the patient. Do not guess or go to another house. Tell the office so they can fix the address in Alora. A wrong address can cause an EVV exception.

▼ **The patient is in the hospital.**

Do not go. Call the office. The office will stop or move the visits. No visits are clocked while the patient is in the hospital.

Clocking in and out

▼ **"Location not found."**

Check that location is on for Alora. Move near a window or a door. Wait 30 seconds and try again. If it still fails, call the office and use the telephone method. See Chapter 8.

▼ **I forgot to clock in or clock out.**

Do not guess or enter a false time. Call the office and tell them the real times. See Chapter 8.

▼ **I clocked in at the wrong patient.**

Stop. Do not continue. Call the office right away. They will correct it and write the reason.

▼ **After I clocked out, the visit does not show Complete.**

Something may be missing, such as a note or a signature. Open the visit and check each part. Or sync may be waiting. If it still does not show Complete, call the office.

Notes and signatures

▼ **My note will not save.**

Read the message on the screen. A required item may be empty. Fill it in. If it still fails, write down the error words, do not delete the app, and call the office. Keep the facts so you can enter them later.

▼ **I wrote a note in the wrong patient's record.**

Tell your supervisor right away. Do not delete it on your own. The note must be corrected, following Zenith's correction process, so that the record keeps a history. It is also a privacy matter, so the office must know.

▼ The signature box does not work.

Close the screen and open it again. Restart the app. If it still fails, do not sign for the patient. Write in the note that the signature could not be collected, and why. Tell your supervisor.

▼ The patient cannot or will not sign.

See Chapter 11. Never sign for the patient. Write the reason and tell your supervisor.

▼ The office sent my note back with a question.

Answer it the same day, honestly, with an addendum. If you do not remember, say so. Do not guess.

Sync and no internet

▼ Items will not leave the waiting list.

Move to a place with better signal. Try again in a minute. Do not log out or delete the app. If it still fails, call the office. See Chapter 9.

At the patient's home

▼ The patient does not answer the door.

Knock again, call the patient, then call the office. If you think the patient may be hurt, call 911 first. Do not clock in unless you give care. See Chapter 7.

▼ I find a patient who does not respond or is not breathing.

Call 911 right away. Stay with the patient. Follow your training. Call the office after you call 911. Tell the 911 worker exactly what you see.

▼ The patient refuses care.

Respect the choice. Explain once why the care matters. Write it down and tell your supervisor.

▼ I feel unsafe in the home.

Leave. Go to a safe place. Call 911 if there is danger. Then call the office. Your safety comes first.

For the office

▼ A team member says they finished a visit, but the Live Monitor shows it as late.

The team member may be offline and not synced yet. Call and ask them to sync. If they cannot, record the real times through the exception review after you ask what really happened.

▼ I found two records for the same patient.

Do not delete either one. Tell your supervisor, so the records can be joined correctly. Do not add new notes to the wrong one.

▼ A fax did not go through.

Check the number digit by digit. Call the doctor's office to confirm the fax number. Send again. If the fax went to the wrong number, report a privacy incident (Chapter 3).

▼ The doctor has not signed the plan of care.

Follow up by phone. Send again. After **7 calendar days**, tell the Director of Nursing. See Chapter 17.

▼ A claim was rejected or denied.

Read the reason, find the cause in the record, and fix the real cause. See Chapter 18. Never change a record just to get paid.

▼ A person left Zenith and still has an account.

Tell the Administrator right away. Former team members must not have access to patient information.

Alora is not working for everyone

▼ Alora is down, or very slow, for the whole team.

Keep caring for patients. Call your supervisor and ask for Zenith's downtime instructions. As good practice, write down the real times and the facts carefully while the system is down. When it works again, enter them as **late entries** with the real date and time. Never make them look like they were written earlier. The office will tell Alora's support team.

☆ **REMEMBER**

- Emergency: call 911 first. Then call the office.
- For any other problem, call your supervisor or the office. Do not guess.
- Do not delete the app or log out while work is waiting to sync.
- Never use another person's account. Never enter a false time.
- When the system is down, keep the facts and enter them later as late entries.

CHECK YOURSELF

? **You cannot log in after two tries. What is the next step?**

Answer: Stop trying and call the office for a reset. More tries can lock the account.

? **The Alora system is down. Are you allowed to skip writing notes until it works?**

Answer: No. Keep the facts carefully, and enter them as late entries with the real date and time when the system returns. Follow Zenith's downtime instructions.

? **You wrote a note in the wrong patient's chart. Do you delete it?**

Answer: No. Tell your supervisor right away. The note must be corrected by Zenith's correction process, and the office must know because it is a privacy matter.

21

Safety, Emergencies, and Reporting Abuse

● Everyone

About 6 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Decide in seconds if a situation is an emergency
- ✓ Protect your own safety in the field
- ✓ Report an incident the right way
- ✓ Report abuse, neglect, or exploitation, as Florida law requires

Nothing in this guide is more important than this chapter. Patients and team members must be safe. When someone is in danger, you do not wait, you do not ask permission, and you do not look in a manual. You act. Read this chapter now, so you know exactly what to do later.

Is it an emergency?

❓ **Is anyone in immediate danger of serious harm or death?**

YES: CALL 911 FIRST

Examples: not breathing, chest pain, serious fall, heavy bleeding, loss of consciousness, a weapon or violence. Call **911**. Stay safe. Stay with the patient if it is safe. Then call the office.

NO: CALL THE OFFICE

Call your supervisor, or the on-call nurse after hours. Tell them what you see. Follow their instructions. If things get worse, call 911.



WHEN IN DOUBT, CALL 911

If you are not sure if it is an emergency, it is better to call. You will never be wrong to get help for someone who may be in danger.

Your own safety



Look at the home before you go in. If you feel unsafe, do not enter.



Park in a safe and lit place. Tell the office if the area is unsafe.



Keep your phone charged and with you, so you can call for help.



Leave at once if there is a threat, a weapon, violence, or drug use that puts you at risk.



Use hand washing and the protective equipment you were trained to use.



Report any needle stick, cut, or contact with blood or body fluid to your supervisor right away.

Incidents

An **incident** is an unexpected event that hurts, or could hurt, a patient or team member. Examples: a fall, a medicine mistake, an injury, a car accident during work, a missing or broken piece of equipment, or a complaint about care.

1

Make everyone safe

Care for the patient first. Call 911 if needed.

2

Call your supervisor

Do it now, the same day, not later. Say what happened, when, and where.

3

Write what happened

Use the incident report form or place that Zenith tells you to use. Ask the office where and how to file it. Write only facts: what you saw, what you did, what the patient said, and who you told.

4

Do not blame, and do not guess

An incident report is for learning and safety. Write what you know. Write "I do not know" for what you do not know.

5

Follow up

Answer questions. Take part in the review. Tell the office if anything changes.

Patient rights

Every patient has rights. You help protect them every day. The federal rules list them. Here are some of the most important.

FEDERAL RULE

Patients have the right to be informed of their rights in a language and manner they understand. (42 CFR 484.50)



To be treated with respect, and to have their person and property respected



To be free from verbal, mental, sexual, and physical abuse, neglect, and stealing of property



To take part in their care, to agree to it, and to refuse it



To receive the services in the plan of care



To have a private clinical record



To make a complaint to Zenith about their care, without fear

If a patient or family member complains to you, listen calmly. Do not argue. Do not promise a result. Tell your supervisor the same day. Every complaint is a chance to make care better.

Abuse, neglect, and exploitation

In Florida, abuse of a vulnerable adult is reported by any person who knows about it, or who has reason to suspect it. A **vulnerable adult** is a person age 18 or older who has trouble with daily activities or with self-protection. The cause can be a disability, an illness, or the infirmities of age. Many of our patients are vulnerable adults.

FLORIDA LAW

Any person who knows, or has reasonable cause to suspect, that a vulnerable adult is being abused, neglected, or exploited **must report it immediately** to the Florida Abuse Hotline. (*Section 415.1034, Florida Statutes*)

Florida Abuse Hotline

1-800-96-ABUSE (1-800-962-2873)

Open 24 hours a day, 7 days a week. You can also use the Florida Relay (7-1-1). Save this number in your phone today.

What to look for



Bruises, burns, cuts, or injuries that cannot be explained



A patient who seems afraid, withdrawn, or who flinches near a person



No food, no heat or water, dirty and unsafe conditions, or no needed care



Missing money, property, or checks. Sudden changes to a will or bank account.



A patient kept from visitors, the phone, or medical care



A patient or other person tells you they are being hurt

What to do

- 1 If the patient is in danger now, call 911**
- 2 Call the Florida Abuse Hotline**

Do this immediately. You do not need proof. You only need a reason to suspect. Give the patient's name and address, what you saw or heard, and who may be responsible.
- 3 Tell your supervisor**

Tell them the same day. The report is still your own duty. You do not need permission from the office to call the Hotline.

4

Write the facts

Write what you saw and heard, the dates and times, and what you did. Do not write opinions.

✓ DO

- ✓ Stay calm and kind with the patient.
- ✓ Report what you saw, as facts.
- ✓ Protect the patient's privacy. Tell only the people who need to know.

✗ DO NOT

- ✗ Do not confront the person you suspect.
- ✗ Do not investigate on your own.
- ✗ Do not wait until you are sure. A suspicion is enough to report.
- ✗ Do not keep it secret to protect a co-worker or a family.

☆ REMEMBER

- If someone may be in danger, call 911 first. Then call the office.
- Leave any home where you feel unsafe.
- Report every incident to your supervisor the same day.
- Florida law: any person must report suspected abuse, neglect, or exploitation of a vulnerable adult, immediately, to 1-800-96-ABUSE.
- You do not need proof or permission to report.

CHECK YOURSELF

? You suspect that a patient's son is taking the patient's money. Do you need proof before you report?

Answer: No. You only need a reason to suspect. Call the Florida Abuse Hotline right away, at 1-800-96-ABUSE, and tell your supervisor the same day.

? Do you need your supervisor's permission to call the Abuse Hotline?

Answer: No. The duty to report belongs to you. Tell your supervisor, but do not wait for permission.

? A patient falls and cannot get up. What do you do first?

Answer: Keep the patient still and safe. Call 911 if the patient may be seriously hurt. Then call your supervisor. Afterwards, file an incident report as Zenith instructs.

22

Honest Records and Compliance

● Everyone

About 5 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Recognize the records that are never allowed
- ✓ Use a simple four-question test before you record anything
- ✓ Know how to report a concern, and that Zenith protects people who do
- ✓ Understand why honest records protect you

Every chapter of this guide comes back to one idea: **the record must be true**. Compliance means following the law and the rules. A compliant record is honest, complete, and on time. This chapter explains why it matters so much, and what to do when you are worried.

WORDS TO KNOW

Compliance

Following the law and the rules.

Fraud

Lying or cheating to get money, for example by billing for care that was not given.

Falsify

To make a record say something that is not true.

Backdate

To write a date that is earlier than the real date.

Why it matters

Medicare and Medicaid pay for care. To protect that money, the law says every record must be true. Health care fraud is a serious crime. It can lead to loss of a license, a fine, and prison, for the person who did it and for the agency. Even a small false entry can cause a large problem. But the opposite is also true: an honest record protects you, the patient, and Zenith.

The never list

These actions are never allowed, for any reason, including when a supervisor or a patient asks.



Clock in or out at a time that is not true



Sign for a patient, or sign for another team member



Use another person's login, or let someone use yours



Copy an old note, or write a note for a visit that did not happen



Backdate or change a record to hide a mistake



Record vital signs or tasks that you did not measure or do



Bill for care that was not given



Pay or accept money or gifts for patient referrals

The four-question test

Before you record, or change, anything, ask these four questions. If any answer is no, stop and ask your supervisor.

1



Is it true?

Did it really happen this way?



2



Is it complete?

Is anything missing that a reader needs?



3



Would I show it?

Would I be comfortable if the patient read it?

4



Would it stand up?

Would it be fine in an audit years from now?

If you make a mistake

Everyone makes mistakes. A mistake is not fraud. Hiding a mistake can be. So:

- Tell your supervisor as soon as you notice.
- Correct it with an addendum or a late entry. Show the real date and time.
- Never delete, never overwrite, and never backdate.

If you are asked to do something wrong

Sometimes a supervisor, a co-worker, a family, or a patient may ask you to write something that is not true, for example to "fix" a time. The answer is always no, politely and clearly.

You can say: "I am sorry, I cannot do that. I have to record what really happened. Can we find another way to solve the problem?"

Then report it. You are not being disloyal. You are protecting the patient and Zenith.

How to report a concern

Every Zenith team member has a duty to report suspected compliance violations. This includes billing errors, falsified records, kickbacks, and problems with exclusion lists. You may report to:

- Your supervisor
- The Administrator
- The Compliance Officer (the name and phone are in Appendix A)

Zenith's compliance commitments also say:

- Reports may be made **anonymously** where an anonymous reporting way is available.
- Zenith **prohibits retaliation**, intimidation, or harassment against anyone who reports a concern in good faith, even if the concern turns out to be wrong. If you think someone is punishing you for a report, report that too.

If you believe that reporting inside Zenith is not right for the situation, you may report outside Zenith. These outside contacts are listed in Zenith's compliance documents:

WHO	FOR WHAT	PHONE
AHCA (Florida Agency for Health Care Administration)	Concerns about a health care agency	1-888-419-3456
HHS Office of Inspector General Hotline	Suspected fraud against Medicare or Medicaid	1-800-HHS-TIPS
Florida Attorney General, Medicaid Fraud Control Unit	Suspected Medicaid fraud	See the Attorney General's website
Florida Abuse Hotline	Abuse, neglect, or exploitation of a vulnerable adult	1-800-96-ABUSE

Records are kept, not destroyed

Never delete or destroy a record, a note, a paper, or a message about a patient. The office keeps records for as long as the law requires. If you think something should be removed, ask your supervisor.

☆ REMEMBER

- Every record must be true, complete, and on time.
- Never: false times, signing for others, shared logins, copied notes, backdating, made-up measurements, or billing for care not given.
- Use the four questions: Is it true? Is it complete? Would I show it? Would it stand up?
- Correct a mistake with an addendum. Never hide it.
- Report concerns to your supervisor, the Administrator, or the Compliance Officer. Retaliation for a good faith report is not allowed.

CHECK YOURSELF

? Your supervisor asks you to change a clock out time so the visit looks longer. What do you do?

Answer: Say no, politely. Explain that you must record what really happened. Then report the request to the Administrator or the Compliance Officer.

? You realize you entered a wrong number in a note last week. What do you do?

Answer: Tell your supervisor, and add a correction with today's real date. Say what was wrong and what is right. A mistake that you fix honestly is not fraud.

? Can you report a concern without giving your name?

Answer: Where Zenith has an anonymous way to report, yes. Also, Zenith prohibits retaliation against anyone who reports in good faith.

APPENDICES

Reference

Use these pages to look things up: Zenith's standards and phone numbers, the glossary, printable cards, the training sign-off, and notes about this guide.

A

Zenith Standards and Contacts

● Everyone

About 4 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Find the phone numbers you need
- ✓ See every Zenith standard in one place
- ✓ Know which items the Administrator must confirm



For the Administrator before this guide is shared. The blank lines and the recommended standards on this page are for Zenith to complete and confirm. Each recommended number is a safe starting point, and Zenith may change it. Change a number in the table, then change the same number where it appears in the chapters (each one has a dotted underline). Then sign at the end of this page.

Zenith Care Home Health, LLC

4699 N State Road 7, Suite J1, Tamarac, FL 33319

Your Alora set-up **ZENITH TO COMPLETE**

ITEM	ZENITH FILLS IN
Mobile app name (Alora Anywhere, Alora Plus, or other)	
Web address for the office (computer)	
Who creates and closes accounts	

ITEM	ZENITH FILLS IN
EVV telephone number (backup method)	_____
How Zenith's EVV data reaches the state system	_____
Alora support contact (for the Administrator)	_____

Important phone numbers

Save these numbers in your phone before your first visit.

WHO	WHEN TO CALL	NUMBER
Emergency	Anyone in danger	911
Florida Abuse Hotline	Suspected abuse, neglect, or exploitation of a vulnerable adult. Open every day, all day.	1-800-96-ABUSE (1-800-962-2873)
Zenith main office	Schedule, clocking, and general questions	_____
Your supervisor	Problems with a patient or a visit	_____
Director of Nursing and Administrator	Clinical questions and serious concerns	_____
Scheduler	Visit changes and call-outs	_____
On-call nurse (after hours and weekends)	Urgent clinical problems	_____
Compliance Officer	Concerns about false records or billing	_____
Privacy contact	Lost device, wrong fax, account misuse	_____
Billing	Claims and payment questions	_____

WHO	WHEN TO CALL	NUMBER
AHCA (Florida Agency for Health Care Administration)	Concerns about a health care agency	1-888-419-3456
HHS Office of Inspector General Hotline	Suspected Medicare or Medicaid fraud	1-800-HHS-TIPS

Zenith standards **ADMINISTRATOR TO CONFIRM**

These are the numbers and times that Zenith sets. Where a law also sets a limit, it is shown, and the Zenith standard must never be longer than the law allows.

NO.	WHAT IT MEANS	LEGAL OR OUTSIDE RULE	RECOMMENDED ZENITH STANDARD	CONFIRMED (INITIALS)
S1	Call the office if you will be later than this for a visit.	No federal time. Patients and the plan of care expect visits on the planned day.	15 minutes	_____
S2	The office checks the Live Monitor and calls a team member when a visit has not started this long after the scheduled start.	No federal time.	15 minutes	_____
S3	The supervisor starts the backup plan (reach the patient, find another team member) when there is no contact this long after the scheduled start.	No federal time.	30 minutes	_____

NO.	WHAT IT MEANS	LEGAL OR OUTSIDE RULE	RECOMMENDED ZENITH STANDARD	CONFIRMED (INITIALS)
S4	Visit notes are written and saved by this time.	Medicare expects records that are timely and accurate. The agency sets the exact time.	the end of the same day	_____
S5	Everything on your phone is synced by this time.	None.	before you finish work for the day	_____
S6	Tell the office about a visit you cannot make.	Plan of care and physician orders set the visit frequency. The office must tell the physician when visits are missed.	as soon as you know, and before the visit starts when possible	_____
S7	Zenith target to finish the comprehensive assessment (OASIS) after the start of care date.	Federal rule: no later than 5 calendar days after the start of care.	3 calendar days	_____
S8	Zenith target to do the recertification assessment.	Federal rule: in the last 5 days of the 60-day period (days 56 to 60).	days 56 to 58	_____
S9	The office reviews EVV exceptions by this time.	State and payer rules can also apply.	1 business day	_____
S10	The office follows up when a plan of care or	The signed plan of care must be	7 calendar days	_____

NO.	WHAT IT MEANS	LEGAL OR OUTSIDE RULE	RECOMMENDED ZENITH STANDARD	CONFIRMED (INITIALS)
	order has not come back signed after this long.	in place before the final claim is billed.		
S11	Answer office messages and calls during your working hours within this time.	None.	2 hours	_____

Approval

I confirm that the Zenith standards and contacts on this page are correct, and that this guide may be used to train Zenith team members.

Administrator name

Signature

Date

Guide edition approved

B

Glossary

● Everyone

About 10 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Look up any word or short form used in this guide

Every word that may be new is explained here in simple language. The list is in alphabetical order. You can also select a word with a dotted underline in any chapter to come directly to its meaning.

A

Addendum Also: Late entry

A new note added to a record after the first note was saved. It explains or adds something. It shows the real date and time it was written. The first note is never erased.

Admission

The day a patient is accepted for Zenith services. The first visit that starts care is called the start of care.

Aggregator

A state system that collects visit proof (EVV data) from many agencies. Alora can send visit data there.

Alora

The computer system Zenith uses to schedule visits, prove visits happened, keep the patient record, and bill. It is cloud-based, which means it runs on the internet.

App Also: Alora Anywhere, Alora Plus

A program on your phone. The Alora mobile app is for field work. Your office

Assessment

A careful check of a patient. The clinician asks questions, looks, and

tells you which Alora app you use.

measures to learn what care the patient needs.

B

Billing period

A block of time for one claim. For Medicare home health it is 30 days.

C

Caregiver

Any person who gives hands-on care to a patient. In this guide we say team member, clinician, or aide so the meaning is clear.

Certification period

A block of up to 60 days covered by one plan of care. After 60 days, the patient is checked again (recertification) if care continues.

Claim

A bill sent to an insurance company or program for care given.

Clinician

A licensed health professional who assesses and treats patients: RN, LPN, PT, OT, ST (speech therapist), or MSW (social worker).

CMS

Centers for Medicare and Medicaid Services. It is the United States government office that runs Medicare and sets many rules for home health.

CMS-485 Also: Plan of care

The standard form for a home health plan of care. Alora can build it from the assessment.

CNA

Certified Nursing Assistant. A trained team member who helps with personal care and basic health tasks. At Zenith, CNAs work in the aide role.

Compliance

Following the law and the rules. A compliant record is honest, complete, and on time.

Conditions of Participation Also: CoP

The federal rules an agency must follow to work with Medicare. They are in Title 42, Part 484, of the Code of Federal Regulations.

Consent

The patient says yes, after understanding what will happen. Consent is given freely and written in the record.

D**Denial**

When a payer refuses to pay a claim. The office can often fix the cause and send the claim again.

Discharge

The end of care with Zenith. The patient may have met goals, moved, chosen to stop, or need a higher level of care.

Documentation

Everything written or recorded about care: notes, vital signs, forms, signatures, and orders.

E**E-fax**

A fax sent from the computer, not from a paper machine. Alora has e-fax built in.

EMR Also: EHR

Electronic Medical Record. The patient's chart, kept on a computer instead of paper. Alora is an EMR. Some people say EHR.

EVV

Electronic Visit Verification. Proof that a visit happened. It records who gave the service, who got it, what service, the date, where, and the start and end times.

Exception

A visit record that does not match what was planned, for example a missing clock out. The office must review it before it is finished.

F

Face-to-face encounter

A doctor visit, or a visit with an allowed practitioner, that Medicare requires for home health. It must be related to why the patient needs home health. It happens up to 90 days before, or up to 30 days after, the start of care.

G

GPS

Global Positioning System. Your phone uses it to find where it is. Alora uses it to record where you clock in and out.

H

HHA Also: Aide

Home Health Aide. A trained team member who helps with personal care such as bathing, dressing, and walking. In this guide, HHA means aide.

HIPAA

The federal law that protects patient health information. It says who may see it and how it must be kept safe.

Homebound

A Medicare term. A homebound patient has great difficulty leaving home, and leaving takes a big effort. The nurse decides this at the first visit.

I

Incident

An unexpected event that hurts, or could hurt, a patient or team member.

Intake

The office steps that bring a new patient into Zenith: receiving the referral, checking details, and getting the orders.

Examples: a fall, a medicine error, a car accident on the way to a visit.

L

Live Monitor

An Alora screen that shows visits as they happen. Colors warn the office about late visits and no-shows.

Log in / Log out

To log in is to enter your username and password to open Alora. To log out is to close your session so nobody else can use your account.

LPN

Licensed Practical Nurse. A licensed nurse who gives nursing care under the direction of an RN or a doctor.

M

Medicaid

A government health program for people with low income. Each state runs its own. In Florida, Medicaid visits must be proved with EVV.

Medicare

The federal health program for people age 65 and older and some younger people with disabilities.

Minimum necessary

A HIPAA idea. Look at, use, and share only the patient information you need to do your job.

MSW

Medical Social Worker. A clinician who helps patients and families with social needs, money worries, and community services.

N

NOA

Notice of Admission. A one-time message to Medicare that a home

NPI

National Provider Identifier. A 10-digit number that identifies a health care

health admission has started. It is due within 5 calendar days after the start of care.

provider, such as a doctor.

O

OASIS

Outcome and Assessment Information Set. A standard set of questions a clinician answers about a patient's health and abilities. Medicare requires it for most home health patients.

Offline mode

Using the app with no internet. The app keeps your work on the phone and sends it later.

Open shift

A visit that has no team member assigned yet. Alora can tell team members that it is available.

OT

Occupational Therapist. A clinician who helps patients do daily activities, such as dressing, cooking, and bathing.

P

Payer

The person or company that pays for care: Medicare, Medicaid, an insurance plan, or the patient.

PDGM

Patient-Driven Groupings Model. The way Medicare pays home health. It groups patients by their needs and pays per 30-day period.

PHI

Protected Health Information. Any information that can identify a patient and relates to health or care: name, address, birthday, diagnosis, photos, and more.

Physician

A doctor. Medicare also allows some other practitioners, such as nurse practitioners, to sign certain orders. In this guide, physician includes them when the law allows.

Physician order Also: Verbal order

A written instruction from a doctor, or allowed practitioner, for care. Care must

Plan of care Also: POC

The written plan for one patient: goals, services, how often, and who does what.

follow the orders.

The doctor signs it. All visits follow it.

Prior authorization

Permission from an insurance company before care is given. Some payers will not pay without it.

PT

Physical Therapist. A clinician who helps patients move, walk, and get stronger.

Q

QA

Quality Assurance. Checking records for mistakes and missing items before they cause a problem.

R

Recertification Also: Recert

A new assessment and review during the last 5 days of each 60-day period (days 56 to 60). It decides whether care continues.

Referral

A request for Zenith to care for a patient. It usually comes from a doctor, hospital, or family.

Remittance

The payer's report that explains what it paid, and what it did not pay, on each claim.

Resumption of care (ROC) Also: ROC

Care that starts again after the patient stayed in a hospital or facility for 24 hours or more. A new assessment is done within 48 hours after the patient returns home.

RN

Registered Nurse. A licensed nurse. An RN does the first assessment visit, writes the plan of care, and supervises aides.

S

Scheduler

The office team member who plans visits and assigns them to team members.

ST

Speech Therapist (speech-language pathologist). A clinician who helps patients with speaking, understanding, and swallowing.

Start of care (SOC) Also: SOC

The date of the first visit that begins care for a patient. The assessment and plan of care count days from this date.

Supervisory visit

A visit or check in which a nurse, or another skilled professional, reviews the aide's work with the patient.

Sync

To send the work saved on your phone to Alora, and to get new information from Alora. Short for synchronize.

T

Task

One thing the plan of care asks you to do during a visit, for example help with bathing.

Telephony EVV

A backup way to clock in and out by calling a phone number. It is for times when the app cannot be used.

U

Username

The name that identifies you in Alora. It belongs only to you.

V

Verbal order

Visit

An order given by speaking, often by phone. You write it in Alora right away. The doctor signs it later.

One time a team member goes to a patient's home to give a service.

Visit frequency

How often a discipline visits, written in the plan of care. Example: 2 visits a week for 4 weeks.

Visit note

The written record of one visit: what was done, what was seen, and how the patient was.

C

Quick Cards and Checklists

● Everyone

About 3 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Find a one-page reminder for the most common tasks
- ✓ Print a card and keep it with you

These cards put the most important steps on one page each. Print them and keep them in your bag or at your desk. Every step is explained in the chapter named on the card.

Card 1. The visit day (Chapters 6 to 9)

- ☐ Before: read the visit card. Charge the phone. Pack your badge and supplies.
- ☐ Arrive. Greet the patient. Wash your hands.
- ☐ Wait for "Location found." Tap **Clock In** once.
- ☐ Give care. Follow the task list.
- ☐ Write the visit note. Facts only. Give reasons for tasks not done.
- ☐ Get the signatures, after care.
- ☐ Tap **Clock Out** at the home. Check the visit shows Complete.
- ☐ Sync. Check the waiting list is empty.

Card 2. Clocking problems (Chapter 8)

PROBLEM	DO THIS
No "Location found"	Location on. Near a window. Wait 30 seconds. Then call the office.
App will not open	Restart. Then use the telephone method and tell the office.
No internet	Clock in anyway. Sync later.
Forgot to clock in or out	Do not guess. Call the office. Tell the real time.
Wrong patient	Stop. Call the office.

Card 3. A good visit note (Chapter 10)

- ☐ Right patient, right date, right visit.
- ☐ Vital signs are real measurements.
- ☐ Every task is marked done or not done, with a reason.
- ☐ I wrote what I saw, what I did, what the patient said, and who I called.
- ☐ I did not copy an old note.
- ☐ I saved it, and I saw the saved message.

Card 4. Who to call (Chapters 20 and 21)

- **Someone may be in danger:** 911 first, then the office.
- **Patient problem or change:** your supervisor or the on-call nurse, now.
- **Late or cannot make a visit:** the office, before the visit if you can.
- **Alora problem:** the office.
- **Lost phone or account problem:** your supervisor, at once.
- **Suspected abuse, neglect, or exploitation:** Florida Abuse Hotline, 1-800-96-ABUSE, at once.

Card 5. The end-of-day check (Chapter 7)

- ☐ Every visit has a clock in and a clock out.
- ☐ Every note is saved.
- ☐ Signatures are done, or the reason is written.
- ☐ Everything is synced.
- ☐ Problems were reported.
- ☐ Tomorrow's schedule is checked. The phone is charging.

Card 6. Clinical deadlines (Chapters 13 and 17)

WHAT	WHEN
Initial assessment visit	Within 48 hours of referral (or the ordered date)
Comprehensive assessment (OASIS)	No later than 5 calendar days after start of care
Notice of Admission (Medicare)	Within 5 calendar days of start of care
Send OASIS to the government system	Within 30 days of completing it
Doctor reviews the plan of care	At least every 60 days
Recertification assessment	Days 56 to 60
Aide supervision	Every 14 days (with skilled care), every 60 days (aide services only)
Therapist reassessment	At least every 30 days

Card 7. The ten golden rules (Chapter 1)

1. Use only your own account.
2. Clock in and out at the patient's home.
3. Write the truth.
4. Write it on time.
5. Patients sign for themselves.
6. Protect privacy.
7. Send your work (sync).
8. Report problems right away.
9. Fix mistakes the right way.
10. Ask when you are not sure.

D

Training Sign-Off

● Everyone

About 2 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Record that you completed your Alora training

Print this page. Complete it with your trainer. Zenith keeps it in your personnel file.

Team member name

Role (RN, LPN, PT, OT, ST, MSW, HHA, CNA, or office)

Trainer name

Date of training

Chapters completed

Mark each chapter that you read and discussed with your trainer.

- ☐ Chapters 1 to 5: the basics (everyone)
- ☐ Chapters 6 to 11: the visit day, clocking, offline work, notes, and signatures (clinicians, aides, and office team)
- ☐ Chapter 12: for aides
- ☐ Chapter 13: for nurses, therapists, and social workers

- ☐ Chapters 14 to 19: the office chapters (office team)
- ☐ Chapters 20 to 22: problems, safety, and honest records (everyone)
- ☐ Practice run completed with my trainer
- ☐ I saved the phone numbers in Appendix A

Acknowledgement

I have read the parts of the Zenith Alora Guide for my role, and I had the chance to ask questions. I understand that:

- ☐ I use only my own Alora account and I keep my password private.
- ☐ Clock times, notes, and signatures must be true, and I will never falsify a record.
- ☐ I must protect patient information.
- ☐ I must report problems, incidents, and suspected abuse, neglect, or exploitation right away.
- ☐ I can ask my supervisor whenever I am not sure.

Team member signature

Date

Trainer signature

Date

E

About This Guide

● Everyone

About 3 minutes to read

IN THIS CHAPTER YOU WILL LEARN TO

- ✓ Understand where the rules and facts in this guide come from
- ✓ See what the Administrator must check before the guide is final

Purpose

This guide gives every Zenith team member one clear place to learn Alora. It explains the steps, the reasons, and the rules. It does not replace Zenith's policies, your training from your supervisor, or legal advice. When a Zenith policy and this guide are different, follow the Zenith policy, and tell the Administrator so the guide can be fixed.

Where the information comes from

- **Federal rules** come from the Medicare Conditions of Participation for home health agencies (Title 42, Part 484, of the Code of Federal Regulations). Sections used: 484.45 (OASIS reporting), 484.50 (patient rights), 484.55 (comprehensive assessment), 484.60 (care planning), and 484.80 (home health aide services).
- **EVV** requirements come from the 21st Century Cures Act. Florida's Medicaid EVV program is described by the Agency for Health Care Administration.
- **Abuse reporting** comes from Section 415.1034, Florida Statutes, and the Florida Abuse Hotline.
- **Alora's features** come from Alora's public product descriptions, for example its EVV, telephony, and home care pages, and the app store listings for Alora Anywhere and Alora Plus.

- **Zenith's own rules** come from Zenith's compliance and human resources documents, and from the standards in Appendix A.

The limits of this edition

 This edition was written without access to Zenith's own Alora account. Statements about Alora describe features that Alora publishes. The drawings are simple pictures. Screen names and button names in Zenith's Alora can be different. The Administrator should check the items below against the live system before this guide is final.

For the Administrator: check against the live system

- ☐ Which mobile app Zenith uses, and the exact name of the clock in, clock out, sync, note, and signature buttons.
- ☐ The exact words on the permission screens and the login screen.
- ☐ How a team member accepts an open visit (in the app, or by calling the scheduler).
- ☐ How the telephone EVV method works for Zenith, and the number to call.
- ☐ How Zenith's EVV data reaches the state EVV system, and how rejected visits are found.
- ☐ The names of the Live Monitor, the exception review, the OASIS check, and the e-fax screens in Zenith's Alora.
- ☐ Whether Zenith requires a patient signature on every visit, and the correct way to record "patient cannot sign."
- ☐ Where to file an incident report, and Zenith's downtime plan for when Alora is not available.
- ☐ The correction, addendum, and late-entry features in Alora, and what they are called.
- ☐ Zenith's approved list of abbreviations.
- ☐ The Zenith standards in Appendix A, and the phone numbers.

How to update this guide

This guide is one file. You can change text directly in the file. Each Zenith standard appears with a dotted underline in the chapters, and is also listed in Appendix A. Change both places. When rules change, such as a new CMS rule, update the chapter and record the change below.

Change log

EDITION	DATE	WHAT CHANGED	APPROVED BY
1	September 2026	First edition.	